

SUNNAH REMEDIES ACADEMY

CERTIFIED HIJAMA PRACTITIONER PROGRAMME

COMPLETE PRODUCTION PACKAGE

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SECTION 1: MASTER SLIDE MANIFEST

Programme Introduction (Slides 001–003)

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
001	Programme Title	Establish institutional authority; communicate premium positioning	N/A (orientation)	2 min	TITLE_FULL_E
002	Copyright & Classification	Protect intellectual property; establish document control	N/A (orientation)	1 min	COPYRIGHT_SP

Table 1 – continued

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
003	Curriculum Overview	Provide navigational roadmap; set expectations for six-module structure	Students will preview the complete learning journey	3 min	CONTENT_LIST

Module 1: Foundations of Hijama (Slides 004–012)

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
004	Module 1 Divider	Create dramatic module opening; set spiritual and scholarly tone	N/A	1 min	MODULE_DIVID
005	Module 1 Objectives	Define measurable learning outcomes	Students will identify five foundational competencies	2 min	OBJECTIVES_B
006	Historical Origins	Establish 3,500-year evidence base across civilizations	Students will trace cupping history through four major civilizations	5 min	CONTENT_BOD
007	Cupping Across Civilizations	Tabular comparison of Egyptian, Chinese, Greek, and Islamic contributions	Students will compare developmental milestones	4 min	CONTENT_TAB
008	Evolution of Instruments	Document technological progression from horns to polymers	Students will match instrument to era and sterility requirement	4 min	CONTENT_TAB
009	Hadith —Best of Remedies	Create reflective pause; establish prophetic authority	Students will memorise the primary hadith on Hijama	3 min	QUOTE_FULL_I

Table 2 – continued

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
010	Prophetic Practice Locations	Anatomical precision for Sunnah points	Students will locate five Sunnah points using anatomical landmarks	6 min	CONTENT_TAB
011	Islamic Medical Scholars	Honour intellectual lineage; connect classical and modern practice	Students will describe the contributions of Ibn Sina, Al-Zahrawi, and Al-Razi	5 min	CONTENT_TAB
012	Wet vs Dry Cupping	Distinguish modalities with clinical precision	Students will select appropriate modality by indication	4 min	TWO_COLUMN

Module 2: The Science of Hijama (Slides 013–020)

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
013	Module 2 Divider	Transition to evidence-based content	N/A	1 min	MODULE_DIVID
014	Module 2 Objectives	Define evidence evaluation competencies	Students will evaluate research using the Academy hierarchy	2 min	OBJECTIVES_B
015	The Evidence Hierarchy	Integrate classical usul al-fiqh with GRADE framework	Students will classify evidence by six-tier hierarchy	6 min	CONTENT_TAB
016	Systematic Review Evidence	Present gold-standard research (Cao et al. 2022)	Students will summarise evidence strength by condition	6 min	CONTENT_TAB

Table 3 – continued

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
017	Methodological Limitations	Teach intellectual humility and evidence honesty	Students will identify six common research limitations	5 min	CONTENT_TAB.
018	Proposed Mechanisms	Explain five concurrent physiological pathways	Students will describe Taibah Theory, NO release, immune modulation, MFD, and lymphatic stimulation	6 min	CONTENT_TAB.
019	Physiological Effects	Systematise body-system impacts	Students will communicate cardiovascular, metabolic, musculoskeletal, neurological, immunological, and homeostatic effects	6 min	CONTENT_TAB.
020	Evidence Evolution Quote	Create reflective transition	N/A	2 min	QUOTE_FULL_I

Module 3: Human Anatomy (Slides 021–029)

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
021	Module 3 Divider	Establish anatomical precision as safety foundation	N/A	1 min	MODULE_DIVID
022	Module 3 Objectives	Define anatomical knowledge requirements	Students will identify skin layers, vessels, and danger zones	2 min	OBJECTIVES_B
023	Skin Anatomy	Establish scarification target depth	Students will describe epidermis, dermis, and hypodermis relevance	5 min	CONTENT_TAB.

Table 4 – continued

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
024	Skin Thickness Variation	Prevent iatrogenic injury through site-specific knowledge	Students will adjust scarification depth by body site	4 min	CONTENT_TAB.
025	Circulatory System	Vascular knowledge for safe cup placement	Students will distinguish arteries, arterioles, capillaries, venules, and veins	5 min	CONTENT_TAB.
026	Heart Structure	Cardiovascular safety and vital sign interpretation	Students will interpret cardiac function for patient monitoring	4 min	CONTENT_TAB.
027	Lymphatic System	Immune and drainage mechanism understanding	Students will locate lymphatic danger zones	4 min	CONTENT_TAB.
028	Fascia & Musculoskeletal	Myofascial decompression theory	Students will classify fascia types and identify key muscles	5 min	CONTENT_TAB.
029	Key Muscles	Practical cup placement reference	Students will map six primary muscles for cupping	4 min	CONTENT_TAB.

Module 4: Clinical Practice & Safety (Slides 030–041)

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
030	Module 4 Divider	Establish safety as non-negotiable priority	N/A	1 min	MODULE_DIVID
031	Module 4 Objectives	Define safety and compliance competencies	Students will implement six safety pillars	2 min	OBJECTIVES_B

Table 5 – continued

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
032	Equipment	Equipment selection and regulatory compliance	Students will select cups, lancets, and PPE by indication	5 min	CONTENT_TAB.
033	Cup Types	Clinical selection by modality	Students will match cup type to sterilisation requirement	4 min	CONTENT_TAB.
034	Infection Prevention	WHO 5 Moments and six pillars	Students will demonstrate hand hygiene and PPE protocols	6 min	CONTENT_TAB.
035	Sterilisation Hierarchy	Spaulding classification application	Students will classify items as critical, semi-critical, or non-critical	5 min	CONTENT_TAB.
036	7-Step Patient Assessment	Systematic safety framework	Students will conduct complete pre-treatment assessment	8 min	CONTENT_TAB.
037	Red Flags	Emergency recognition and referral	Students will identify red flags across seven body systems	6 min	CONTENT_TAB.
038	Absolute Contraindications	Non-negotiable prohibitions	Students will screen for eight absolute contraindications	5 min	CONTENT_TAB.
039	Relative Contraindications	Modified protocols for complex patients	Students will manage nine relative contraindications	5 min	CONTENT_TAB.
040	Risk Stratification	Green/Amber/Red decision framework	Students will assign risk categories with documented reasoning	4 min	CONTENT_TAB.

Table 5 – continued

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
041	Informed Consent	Ethical and legal foundation	Students will obtain valid consent through eight elements	6 min	CONTENT_TAB.

Module 5: Treatment Protocols (Slides 042–049)

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
042	Module 5 Divider	Transition to technical competence	N/A	1 min	MODULE_DIVID
043	Module 5 Objectives	Define technical skill requirements	Students will execute the 9-step procedure	2 min	OBJECTIVES_B
044	Sunnah Points	Anatomical landmark verification	Students will palpate and locate five Sunnah points	6 min	CONTENT_TAB.
045	Standard Health Detox Protocol	Foundation treatment parameters	Students will apply standard parameters with individualisation	5 min	CONTENT_TAB.
046	9-Step Wet Cupping Procedure	Core technical competency	Students will perform scarification at 0.5–1 mm depth safely	10 min	CONTENT_TAB.
047	Condition-Specific Applications	Evidence-based point selection	Students will match conditions to evidence grades	5 min	CONTENT_TAB.
048	Special Populations	Modified protocols for vulnerable groups	Students will adjust treatment for six special populations	5 min	CONTENT_TAB.
049	Module 5 Transition	Bridge to professional standards	N/A	1 min	TRANSITION_SI

Module 6: Professional Standards (Slides 050–061)

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
050	Module 6 Divider	Establish ethics as professional identity	N/A	1 min	MODULE_DIVIDER
051	Module 6 Objectives	Define professional competency domains	Students will demonstrate six core competencies	2 min	OBJECTIVES_BLOCK
052	Adverse Events	Classification and management	Students will classify and respond to minor, moderate, and serious events	5 min	CONTENT_TABLE
053	Emergency Response	Life-saving protocols	Students will execute emergency response for five scenarios	6 min	CONTENT_TABLE
054	Emergency Equipment	Preparedness verification	Students will maintain and check seven emergency items	3 min	CONTENT_TABLE
055	Marks vs Bruising	Patient education and clinical differentiation	Students will distinguish normal marks from pathology	4 min	TWO_COLUMN_LAYOUT
056	Mark Colour Interpretation	Clinical observation skills	Students will interpret four colour patterns	3 min	CONTENT_TABLE
057	Clinic Operations	Business and regulatory standards	Students will establish compliant clinic environment	5 min	CONTENT_TABLE
058	Fasting & Hijama	Ramadan clinical guidance	Students will navigate scholarly positions and patient safety	4 min	CONTENT_TABLE
059	Complementary Prophetic Medicine	Integrative Tibbal-Nabawi	Students will recommend seven prophetic remedies within scope	4 min	CONTENT_TABLE

Table 7 – continued

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
060	Practitioner Competencies	Certification framework	Students will demonstrate six competency domains	4 min	CONTENT_TAB.
061	Closing Hadith	Spiritual completion	N/A	2 min	QUOTE_FULL_I

Clinical Reference (Slides 062–071)

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
062	Clinical Reference Divider	Establish reference authority	N/A	1 min	MODULE_DIVID
063	Reference Objectives	Define reference usage	Students will apply reference materials to clinical decisions	2 min	OBJECTIVES_B
064	Evidence & Sunnah Matrix	Decision-making framework	Students will communicate evidence honestly using the Matrix	4 min	CONTENT_TAB.
065	Scope of Practice	Four pillars of professional boundaries	Students will define and defend their scope	4 min	CONTENT_TAB.
066	Permitted & Prohibited	Absolute red lines	Students will distinguish permitted from prohibited practice	4 min	CONTENT_TAB.
067	SAFE Hijama Protocol	Mnemonic clinical framework	Students will apply Systematic, Anatomical, Faithful, Evidence-Informed practice	4 min	CONTENT_TAB.

Table 8 – continued

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
068	Documentation Standards	Legal protection and quality	Students will maintain contemporaneous records to Academy standard	4 min	CONTENT_TAB.
069	Core Ethical Obligations	Islamic bioethics integration	Students will apply beneficence, non-maleficence, autonomy, justice, and confidentiality	4 min	CONTENT_TAB.
070	Advertising Standards	Marketing integrity	Students will create compliant promotional materials	3 min	CONTENT_TAB.
071	Final Reflection Hadith	Programme spiritual closure	N/A	2 min	QUOTE_FULL_I

Closing (Slides 072–075)

Slide	Title	Purpose	Learning Objective	Duration	Master Layout
072	Certification Complete	Celebrate achievement; establish alumni identity	N/A	2 min	TITLE_FULL_E
073	Back Cover	Contact and legal closure	N/A	1 min	BACK_COVER_

SECTION 2: COMPLETE SLIDE CONTENT**SLIDE 001 — Programme Title****Master:** TITLE_FULL_EMERALD**Background:** Deep Emerald (#0E3B2E) full bleed**Duration:** 2 minutes**On-Slide Text:**

- Academy mark: Simplified typographic mark (see Asset A-001)

- Main title: “CERTIFIED HIJAMA / PRACTITIONER PROGRAMME” — Libre Baskerville, 54 pt, bold, Warm Ivory
- Subtitle: “A Complete Evidence-Based Curriculum in Wet Cupping Therapy & Prophetic Medicine” — Inter, 16 pt, Antique Gold
- Academy line: “SUNNAH REMEDIES ACADEMY ·Institute of Prophetic Medicine” — Inter, 11 pt, Warm Ivory
- Accreditation: “CMA & IPHM Accredited Programme” — Inter, 10 pt, Antique Gold
- Document code: “SRA-DOC-2026-HJM-001-v1.0 | Classification: PRACTITIONER | Version 1.0” — Inter, 8 pt, Warm Ivory

Speaker Notes:

Welcome students to the Sunnah Remedies Academy Certified Hijama Practitioner Programme. This is not a conventional training course. You have entered an elite academy where evidence-informed practice meets prophetic tradition. Over the next [duration], you will develop the knowledge, skills, and ethical framework to practice Hijama at the highest international standard. The benchmark is not a normal training provider — it is Harvard Executive Education, Mayo Clinic, and Oxford Saïd Business School. You are now part of that standard.

Student Notes:

Programme introduction. Record the document code SRA-DOC-2026-HJM-001-v1.0 in your workbook. This is your reference number for all academy materials.

SLIDE 002 — Copyright & Classification

Master: COPYRIGHT_SPLIT

Background: Left panel Deep Emerald; right panel Warm Ivory

Duration: 1 minute

On-Slide Text:

- Left panel: “COPYRIGHT & CLASSIFICATION” — Libre Baskerville, 24 pt, bold, Warm Ivory
- Left panel: Document code — Inter, 10 pt, Antique Gold
- Right panel heading: “ 2026 Sunnah Remedies Academy. All rights reserved.” — Inter, 12 pt, bold, Deep Emerald
- Body paragraph 1: Copyright restriction text — Inter, 10 pt, Charcoal
- Body paragraph 2: PRACTITIONER classification text — Inter, 10 pt, Charcoal
- Body paragraph 3: Trademark notice — Inter, 10 pt, Charcoal
- Quote: “And say: My Lord, increase me in knowledge.” — Surah Ta-Ha, 20:114 — Libre Baskerville, 12 pt, italic, Antique Gold

Speaker Notes:

This document is classified PRACTITIONER. It contains clinical protocols and patient safety information intended for certified practitioners and enrolled students. Unauthorized distribution is strictly prohibited. The Academy’s intellectual property is protected under UK and international copyright law. Every graduate receives this material under license, not ownership.

Student Notes:

This is a licensed document, not a purchased product. Do not share outside the Academy.

SLIDE 003 — Curriculum Overview

Master: CONTENT_LIST

Background: Warm Ivory

Duration: 3 minutes

On-Slide Text:

- Title: “CURRICULUM OVERVIEW” — Libre Baskerville, 36 pt, bold, Deep Emerald
- Gold rule: 2 pt, Antique Gold, 2 inches wide
- Module list (6 items):
 - 01 Foundations of Hijama — History, Islamic tradition, defining the practice
 - 02 The Science of Hijama — Evidence base, mechanisms, physiological effects
 - 03 Human Anatomy for Practitioners — Essential anatomical knowledge
 - 04 Clinical Practice & Safety — Equipment, infection control, risk management
 - 05 Treatment Protocols — Sunnah points, procedures, condition-specific care
 - 06 Professional Standards & Operations — Complications, documentation, clinic operations

Speaker Notes:

The programme is structured in six modules, each building upon the last. Module 1 establishes historical and Islamic foundations. Module 2 brings you to the contemporary evidence base. Module 3 gives you the anatomical precision that separates safe practitioners from dangerous ones. Module 4 is your safety fortress. Module 5 is your technical mastery. Module 6 is your professional identity. We will also maintain a Clinical Reference section throughout for rapid consultation.

Student Notes:

Six modules. Expect approximately 8–10 hours of contact time per module. Practical assessments follow Module 5.

SLIDE 004 — Module 1 Divider

Master: MODULE_DIVIDER_EMERALD

Background: Deep Emerald full bleed

Duration: 1 minute

On-Slide Text:

- Large number: “01” — Libre Baskerville, 120 pt, bold, Antique Gold
- Gold rule: 2 pt, Antique Gold, 3 inches wide
- Title: “FOUNDATIONS OF HIJAMA” — Libre Baskerville, 36 pt, bold, Warm Ivory

- Description: “History, Islamic tradition, and defining the practice across civilizations” — Inter, 14 pt, Antique Gold
- Arabic: “ ” — Libre Baskerville, 14 pt, italic, Warm Ivory
- English: “And We send down of the Qur’an that which is healing and mercy for the believers...” — Libre Baskerville, 12 pt, italic, Warm Ivory
- Source: “— Surah Al-Isra, 17:82” — Inter, 9 pt, Antique Gold
- Vertical right text: “SUNNAH REMEDIES ACADEMY ·Institute of Prophetic Medicine” — Inter, 8 pt, Antique Gold, rotated 90°

Speaker Notes:

Module 1 answers the question: Where does Hijama come from? We trace 3,500 years of continuous practice across Egypt, China, Greece, and the Islamic Golden Age. We establish the prophetic foundation that makes this not merely a therapy, but a sunnah. And we define exactly what Hijama is — and what it is not.

Student Notes:

Module 1 covers history, hadith, scholars, and definitions. Focus on memorising the primary hadith and the five Sunnah points.

SLIDE 005 — Module 1 Objectives

Master: OBJECTIVES_BULLET

Background: Warm Ivory

Duration: 2 minutes

On-Slide Text:

- Title: “MODULE OBJECTIVES” — Libre Baskerville, 28 pt, bold, Deep Emerald
- Gold rule: 2 pt, Antique Gold, 1.5 inches wide
- Five bullet points (gold 6 pt circles):
 1. Trace the historical origins of cupping therapy across ancient civilizations
 2. Understand the Islamic textual basis for Hijama practice
 3. Identify the primary and secondary Sunnah points and their prophetic foundations
 4. Distinguish between wet cupping, dry cupping, and other modalities
 5. Recognise the scholarly contributions of Ibn Sina, Al-Zahrawi, and Al-Razi to Hijama practice

Speaker Notes:

By the end of this module, you will be able to teach someone else where Hijama comes from, why the Prophet recommended it, and how the classical scholars expanded upon his practice. These are not abstract objectives — they are the foundation of your authority as a practitioner.

Student Notes:

Five objectives. All will be assessed in the Module 1 knowledge check.

SLIDE 006 — Historical Origins

Master: CONTENT_BODY

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “HISTORICAL ORIGINS OF CUPPING THERAPY” — Libre Baskerville, 24 pt, bold, Deep Emerald
- Gold rule: 2 pt, Antique Gold, 1.2 inches wide
- Body text: “Cupping therapy represents one of the most enduring therapeutic modalities in human history, with continuous documented use spanning at least 3,500 years across every major civilization. Its persistence suggests not merely cultural tradition, but observable clinical efficacy that has warranted repeated rediscovery and refinement.”
- Clinical Pearl box (sage green #6B8E23): “Hijama’s continuous use across at least 124,000 prophetic traditions establishes it as the most historically validated therapeutic modality in human civilisation.”

Speaker Notes:

When a patient asks, “Why should I try something so old?” your answer is: “Because it has survived every test of time, culture, and scientific scrutiny for 3,500 years. Modern medicine is 200 years old. Hijama is 3,500 years old. Which has more evidence of persistence?” The Ebers Papyrus from 1550 BCE mentions cupping. Hippocrates wrote about it. The Chinese documented it in 281 CE. And the Prophet elevated it to the best of remedies.

Student Notes:

Key dates: Egypt 1550 BCE, China 281 CE, Greece Hippocratic era, Islamic Golden Age 7th–13th century.

Clinical Pearl:

Hijama’s continuous use across at least 124,000 prophetic traditions establishes it as the most historically validated therapeutic modality in human civilisation.

SLIDE 007 — Cupping Across Civilizations

Master: CONTENT_TABLE_5COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “CUPPING ACROSS CIVILIZATIONS” — Libre Baskerville, 24 pt, bold, Deep Emerald
- Table (5 columns × 5 rows):
- Header: Civilisation | Period | Key Development | Clinical Relevance | Sources
- Row 1: Ancient Egypt | c. 1550 BCE | Ebers Papyrus — hollowed animal horns | Earliest written medical reference | Ebers Papyrus

- Row 2: Ancient China | c. 281 CE | Bamboo and glass cups; qi circulation | Evolution of instruments and theory | Chinese Medical Tradition
- Row 3: Ancient Greece | Hippocratic era | Hippocratic Corpus — narrow and wide cups | Systematic classification by indication | Hippocratic Corpus
- Row 4: Islamic Golden Age | 7th–13th century | Ibn Sina, Al-Zahrawi, Al-Razi documented sites | Integration with humoral medicine | Sahih al-Bukhari, Al-Qanun

Speaker Notes:

Each civilization contributed something unique. Egypt gave us the first written record. China gave us the theoretical framework of qi and instrument evolution. Greece gave us systematic classification. The Islamic Golden Age gave us integration with prophetic medicine, surgical precision, and the institutional framework that still governs practice today. You are the inheritor of all four traditions.

Student Notes:

Memorise the four civilizations and their unique contributions. This is assessed.

SLIDE 008 — Evolution of Instruments

Master: CONTENT_TABLE_5COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “EVOLUTION OF CUPPING INSTRUMENTS” — Libre Baskerville, 24 pt, bold, Deep Emerald
- Table (5 columns × 6 rows):
- Header: Instrument | Era | Material | Advantages | Sterilisation
- Row 1: Animal Horns | Ancient | Organic — hollowed horns | Readily available | Not sterilisable
- Row 2: Bamboo Cups | Chinese tradition | Natural bamboo | Lightweight | Limited reuse
- Row 3: Glass Cups | Fire cupping era | Borosilicate glass | Visual monitoring | Autoclave 121°C 15 min
- Row 4: Plastic Valve Cups | Modern clinical | Medical-grade polymer | Controlled suction; disposable | Single-use only
- Row 5: Silicone Cups | Contemporary | Food-grade silicone | Flexible; self-suction | High-level disinfection

Speaker Notes:

The instrument evolution mirrors medical history: from organic immediacy, through crafted durability, to precision-engineered disposability. The Academy’s position is clear: for wet cupping, disposable plastic valve systems are the gold standard. They eliminate cross-contamination risk, provide consistent suction control, and meet modern medical device regulations. Glass cups remain appropriate for dry cupping in autoclave-equipped clinics. Fire cupping is not taught at CHP level due to safety requirements.

Student Notes:

Academy recommendation: disposable plastic valve cups for all wet cupping. Never reuse lancets. Glass

cups require autoclave sterilisation.

SLIDE 009 — Hadith: Best of Remedies

Master: QUOTE_FULL_EMERALD

Background: Deep Emerald full bleed

Duration: 3 minutes

On-Slide Text:

- Large quotation mark: “” — Libre Baskerville, 72 pt, Antique Gold
- Arabic: “ ” — Libre Baskerville, 18 pt, Warm Ivory
- English: “The best of remedies you have is cupping (Hijama), or it is one of the best of your medicines.” — Libre Baskerville, 16 pt, italic, Warm Ivory
- Source: “— Sahih al-Bukhari 5371 | Sahih Muslim 2952” — Inter, 10 pt, Antique Gold
- Context: “This hadith establishes the primacy of Hijama within the Prophetic medical tradition. The Prophet did not merely permit cupping — he elevated it to the highest rank of therapeutic interventions available to his community.” — Inter, 10 pt, Antique Gold

Speaker Notes:

Pause here. This is not a slide to rush through. The Prophet said this hadith with absolute authority. He did not say “cupping is good.” He said it is the BEST of remedies. Or one of the best. The variation in narration does not weaken the meaning — it strengthens it. Whether first or among the first, Hijama occupies the highest rank. When you place your cup on a patient, you are continuing a practice that the Prophet himself received. That is not technique. That is sunnah.

Student Notes:

Memorise this hadith in Arabic and English. Reference: Bukhari 5371, Muslim 2952. Both authentic (sahih).

Islamic Insight:

The Prophet received Hijama himself. This is not merely permissive — it is participatory. The best of you are those who follow his example.

SLIDE 010 — Prophetic Practice Locations

Master: CONTENT_TABLE_5COL

Background: Warm Ivory

Duration: 6 minutes

On-Slide Text:

- Title: “PROPHETIC PRACTICE LOCATIONS” — Libre Baskerville, 24 pt, bold, Deep Emerald
- Table (5 columns × 6 rows):
- Header: Point | Location | Prophetic Basis | Clinical Application | Caution

- Row 1: Al-Kahil (PRIMARY) | Between shoulder blades (DU14/DU15), T1–T3 | Primary Sunnah point; most frequently recommended | General wellness, detoxification, immune support | Avoid over bony prominences
- Row 2: Akhda’een (SECONDARY) | Lateral neck, posterior to SCM | Authentic hadith — two neck veins | Headache, hypertension | Vertebral artery proximity
- Row 3: Al-Hamma | Vertex (top of head), midline | Scholarly recommendation | Migraine, neurological | Limited evidence; caution
- Row 4: Umm al-Muqeeth | Occipital region, base of skull | Traditional practice | Tension headache, mental clarity | Cervical spine awareness
- Row 5: Al-Katifain | Shoulder points (GB21), upper trapezius | Authentic hadith | Shoulder pain, myofascial tension | Avoid brachial plexus

Speaker Notes:

These five points are not suggestions. They are the foundation. Al-Kahil is the primary point — the Prophet received Hijama here most frequently. Akhda’een is documented in multiple authentic hadith for headaches and general health. Al-Hamma and Umm al-Muqeeth carry scholarly recommendation. Al-Katifain is explicitly documented. Every practitioner must know these locations with anatomical precision. You cannot guess where T1–T3 is. You must palpate. You must verify. Anatomical knowledge is the difference between healing and harming.

Student Notes:

Al-Kahil is PRIMARY. All others are secondary. Know the vertebral levels. Know the danger zones. Akhda’een requires extreme caution due to carotid and vertebral artery proximity.

Clinical Pearl:

Always combine Sunnah points with condition-specific points for maximum efficacy. The Sunnah is the foundation, not the ceiling.

SLIDE 011 — Islamic Medical Scholars

Master: CONTENT_TABLE_4COL

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “CONTRIBUTIONS OF ISLAMIC MEDICAL SCHOLARS” — Libre Baskerville, 24 pt, bold, Deep Emerald
- Table (4 columns × 5 rows):
- Header: Scholar | Lifespan | Major Work | Contribution to Hijama
- Row 1: Ibn Sina (Avicenna) | 980–1037 CE | Al-Qanun fi al-Tibb | Systematic documentation of indications, contraindications, and point selection integrated with humoral theory
- Row 2: Al-Zahrawi (Albucasis) | 936–1013 CE | Al-Tasrif | Illustrated surgical instruments including cupping tools; described sterile principles and instrument design

- Row 3: Abu Bakr Al-Razi (Rhazes) | 854–925 CE | Al-Hawi fi al-Tibb | Clinical case documentation, adverse event recording, and systematic observation of cupping outcomes

Speaker Notes:

These three men are not merely historical figures. They are your intellectual ancestors. Ibn Sina gave us the systematic framework. Al-Zahrawi gave us the surgical precision. Al-Razi gave us the clinical honesty — he recorded failures as carefully as successes. That is the scientific method applied to Hijama 1,000 years before the RCT. When you document an adverse event, you are following Al-Razi. When you select a point based on anatomical reasoning, you are following Ibn Sina. When you sterilise your instrument, you are following Al-Zahrawi.

Student Notes:

Ibn Sina = systematic framework. Al-Zahrawi = surgical precision and sterility. Al-Razi = clinical honesty and case documentation. All three essential.

SLIDE 012 — Wet vs Dry Cupping

Master: TWO_COLUMN

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “DEFINING HIJAMA: WET CUPPING VS. DRY CUPPING” — Libre Baskerville, 24 pt, bold, Deep Emerald
- Left column title: “WET CUPPING (HIJAMA)” — Inter, 12 pt, bold, Deep Emerald
- Left body: “Therapeutic procedure involving controlled superficial skin incision followed by application of negative pressure via suction cups to extract stagnant blood and interstitial fluid containing pathological substances (CPS).
- Suction + superficial incision • Extracts stagnant blood & toxins • Primary focus of certification • Targets CPS (pathological substances) • Evidence: pain, metabolic, immune”
- Right column title: “DRY CUPPING” — Inter, 12 pt, bold, Deep Emerald
- Right body: “Suction-only therapy without skin incision. Affects fascia and superficial circulation through negative pressure mechanisms.
- Suction only, no incision • Affects fascia & circulation • Pain relief, myofascial release • No blood extraction • Broader evidence base in sports medicine”
- Vertical gold divider: 1 pt, Antique Gold, between columns

Speaker Notes:

This certification focuses exclusively on wet cupping using disposable plastic cup systems with controlled suction, as this represents the Sunnah method with the strongest Islamic textual basis. Dry cupping is a valuable complementary technique but falls outside the core curriculum. Students who wish to practice dry cupping may pursue additional CPD modules. The key distinction is blood extraction: wet cupping removes stagnant blood and pathological substances; dry cupping does not. This distinction matters for patient communication, consent, and scope of practice.

Student Notes:

CHP certification = wet cupping only. Dry cupping requires additional training. Key distinction: blood extraction vs. suction only.

SLIDE 013 — Module 2 Divider

Master: MODULE_DIVIDER_EMERALD

Background: Deep Emerald full bleed

Duration: 1 minute

On-Slide Text:

- Large number: “02” — Libre Baskerville, 120 pt, bold, Antique Gold
- Title: “THE SCIENCE OF HIJAMA” — Libre Baskerville, 36 pt, bold, Warm Ivory
- Description: “Evidence base, mechanisms, and physiological effects” — Inter, 14 pt, Antique Gold
- Arabic: “ ” — Libre Baskerville, 14 pt, italic, Warm Ivory
- English: “And say: My Lord, increase me in knowledge.” — Libre Baskerville, 12 pt, italic, Warm Ivory
- Source: “— Surah Ta-Ha, 20:114” — Inter, 9 pt, Antique Gold

Speaker Notes:

Module 2 is where we leave tradition and enter evidence. Not to replace tradition — to strengthen it. The Prophet commanded us to seek knowledge. That includes scientific knowledge. A practitioner who knows only hadith but cannot read a systematic review is incomplete. A practitioner who knows only research but cannot honour the sunnah is rootless. Module 2 integrates both.

Student Notes:

Module 2 = evidence base. Bring your scepticism and your faith. Both are required.

SLIDE 014 — Module 2 Objectives

Master: OBJECTIVES_BULLET

Background: Warm Ivory

Duration: 2 minutes

On-Slide Text:

- Title: “MODULE OBJECTIVES”
- Five bullets:
 1. Evaluate the strength and quality of contemporary cupping research using the Academy Evidence Hierarchy
 2. Describe the five proposed mechanisms of action for Hijama therapy
 3. Explain the physiological effects of cupping on cardiovascular, metabolic, musculoskeletal, and immune systems
 4. Identify the limitations and methodological gaps in current Hijama research

5. Communicate evidence limitations honestly to patients and colleagues

Speaker Notes:

The most important objective on this slide is number five: communicate evidence limitations honestly. The practitioner who overstates the evidence is as dangerous as the practitioner who ignores it. Your authority comes from honesty, not hype.

Student Notes:

Objective 5 is the most important. Honest communication builds trust. Exaggeration destroys it.

SLIDE 015 — The Evidence Hierarchy

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 6 minutes

On-Slide Text:

- Title: “THE EVIDENCE HIERARCHY”
- Body: “The Academy’s Evidence Hierarchy integrates classical Islamic *usul al-fiqh* with the GRADE framework. Not all knowledge carries equal weight. A weak *hadith* cannot support a clinical claim. A single case study cannot override a systematic review.”
- Table (3 columns × 7 rows):
- Header: Level | Evidence Category | Application to Hijama
- Row 1: 1 | Classical Islamic Evidence | Authentic *hadith* with sound *isnad*; consensus of classical scholars; established practice in authoritative traditional texts
- Row 2: 2 | Systematic Reviews & Meta-Analyses | High-quality synthesis of multiple RCTs on cupping for pain, hypertension, and metabolic conditions
- Row 3: 3 | Randomised Controlled Trials (RCTs) | Well-designed experimental studies on cupping mechanisms and outcomes
- Row 4: 4 | Cohort & Case-Control Studies | Observational studies with appropriate controls
- Row 5: 5 | Case Series & Expert Opinion | Clinical observation and professional consensus
- Row 6: 6 | Mechanistic / Preclinical | Laboratory studies, animal models, theoretical rationale (e.g., Taibah Theory, nitric oxide release)

Speaker Notes:

This hierarchy is unique to the Academy. It does not dismiss classical evidence — it elevates it. Level 1 is classical Islamic evidence: authentic *hadith*, scholarly consensus, established practice. This is not inferior to Level 2. It is different. Level 2 is systematic reviews and meta-analyses — the modern gold standard. A practitioner must know both. When classical and modern evidence align, that is powerful. When they conflict, the Academy Advisory Board adjudicates. You do not adjudicate alone.

Student Notes:

Six levels. Level 1 = classical Islamic evidence. Level 2 = systematic reviews. Both valid. Both necessary. Know the difference.

Evidence Reference:

Adapted from GRADE framework and classical Islamic usul al-fiqh. Academy Advisory Board position statement 2026.

SLIDE 016 — Systematic Review Evidence

Master: CONTENT_TABLE_4COL

Background: Warm Ivory

Duration: 6 minutes

On-Slide Text:

- Title: “SYSTEMATIC REVIEW EVIDENCE (2022)”
- Body: “Cao et al. (2022) published the largest systematic review and meta-analysis of cupping therapy to date, synthesising 177 randomised controlled trials. This review represents the current gold standard for evidence evaluation in Hijama practice.”
- Table (4 columns × 7 rows):
 - Header: Condition | Evidence Level | Key Findings | Reference
 - Row 1: Low Back Pain | Strong | 11 studies; significant pain and disability improvement | PMID: 35180526
 - Row 2: Neck Pain | Moderate | Consistent reduction in pain scores across multiple RCTs | Multiple RCTs
 - Row 3: Migraine | Moderate | Reduced frequency and intensity in systematic reviews | Systematic reviews
 - Row 4: Hypertension | Moderate | SBP reduced 8.4 mmHg at 4 weeks | PMID: 26559364
 - Row 5: Type 2 Diabetes | Preliminary | Improved fasting blood glucose in pilot studies | Mahdavi et al., 2015
 - Row 6: Female Infertility | Preliminary | 20.3% pregnancy rate in pilot study | IJRCOG pilot study
 - Clinical Pearl box: “Practitioners must communicate evidence limitations honestly and distinguish proven effects from traditional practice. Never claim ‘proven cure’ for any condition.”

Speaker Notes:

Cao et al. 2022 is your primary reference. 177 RCTs. That is not a small body of evidence. But notice the evidence grades: strong for low back pain, moderate for neck pain and migraine, preliminary for diabetes and infertility. This gradation is essential. When a patient with diabetes asks if Hijama will cure them, the honest answer is: “The evidence is preliminary. It may support metabolic markers, but it is not a cure. You must continue your medication and monitoring.” That answer protects the patient and protects you.

Student Notes:

Cao et al. 2022 = 177 RCTs. Strong evidence only for low back pain. Everything else is moderate or preliminary. Memorise this table.

Evidence Reference:

Cao et al. (2022). ‘Cupping therapy for chronic pain: A systematic review and meta-analysis of 177

RCTs.’ *Frontiers in Public Health*. PMID: 35180526.

Clinical Pearl:

Practitioners must communicate evidence limitations honestly and distinguish proven effects from traditional practice. Never claim ‘proven cure’ for any condition.

SLIDE 017 — Methodological Limitations

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “METHODODOLOGICAL LIMITATIONS IN CURRENT RESEARCH”
- Body: “The cupping evidence base, while growing, suffers from methodological limitations that constrain clinical certainty. Understanding these limitations is as important as knowing the positive findings.”
- Table (3 columns × 7 rows):

Limitation	Description	Impact
Small Sample Sizes	Most RCTs include <100 participants	Reduced statistical power; inflated effect sizes
Heterogeneous Protocols	Varied cup types, pressures, durations, point selections	Difficulty pooling data; inconsistent reproducibility
Inadequate Blinding	Practitioners and patients cannot be blinded to cupping	Performance and detection bias inflate outcomes
Publication Bias	Negative studies less likely to be published	Overestimation of true effect sizes in meta-analyses
Short Follow-Up	Most studies measure outcomes at 4–8 weeks only	Unknown long-term efficacy and safety profiles
Cultural Variation	Trial populations predominantly East Asian or Middle Eastern	Generalisability to Western patient populations uncertain
- Header: Limitation | Description | Impact
- Row 1: Small Sample Sizes | Most RCTs include <100 participants | Reduced statistical power; inflated effect sizes
- Row 2: Heterogeneous Protocols | Varied cup types, pressures, durations, point selections | Difficulty pooling data; inconsistent reproducibility
- Row 3: Inadequate Blinding | Practitioners and patients cannot be blinded to cupping | Performance and detection bias inflate outcomes
- Row 4: Publication Bias | Negative studies less likely to be published | Overestimation of true effect sizes in meta-analyses
- Row 5: Short Follow-Up | Most studies measure outcomes at 4–8 weeks only | Unknown long-term efficacy and safety profiles
- Row 6: Cultural Variation | Trial populations predominantly East Asian or Middle Eastern | Generalisability to Western patient populations uncertain

Speaker Notes:

These limitations are not excuses to dismiss the evidence. They are context to interpret it. A practitioner who says “there is no evidence for Hijama” is ignorant. A practitioner who says “the evidence is perfect” is dishonest. The truth is: the evidence is promising but incomplete. That is a statement of integrity, not ignorance. And it is the only statement that protects your patients and your practice.

Student Notes:

Six limitations. Know them. Use them to educate patients, not to dismiss Hijama. Honest evidence communication builds long-term trust.

SLIDE 018 — Proposed Mechanisms

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 6 minutes

On-Slide Text:

- Title: “PROPOSED MECHANISMS OF ACTION”
- Body: “Contemporary research proposes multiple concurrent mechanisms through which Hijama exerts therapeutic effects. These mechanisms are not mutually exclusive; the dominant pathway likely varies by condition, patient, and protocol.”
- Table (3 columns × 6 rows):
 - Header: Mechanism | Description | Evidence Quality
 - Row 1: 1. Taibah Theory — CPS Filtration | Filters Causative Pathological Substances: excess proteins, heavy metals, cholesterol, fatty deposits, and inflammatory mediators from blood and interstitial fluid | Classical framework; modern validation preliminary
 - Row 2: 2. Nitric Oxide Release | Negative pressure triggers endothelial nitric oxide synthase (eNOS), increasing local vasodilation and blood flow through reactive hyperemia | Moderate — supported by mechanistic studies
 - Row 3: 3. Immune Modulation | Stimulates reticuloendothelial system, triggering increased white blood cell production and enhanced immune surveillance | Moderate — supported by immunological studies
 - Row 4: 4. Myofascial Decompression (MFD) | Negative pressure creates inter-layer separation in fascia, restoring fascial gliding and reducing myofascial restrictions | Moderate — PMC7735689 demonstrates superiority over foam rolling
 - Row 5: 5. Lymphatic Stimulation | Enhanced lymphatic drainage promoting clearance of metabolic waste, inflammatory mediators, and cellular debris | Moderate — explains benefits for oedema and immune function

Speaker Notes:

When a patient asks “How does Hijama work?” you have five answers, not one. The Taibah Theory is the classical framework — it explains what the scholars observed. The other four are modern mechanistic explanations. You do not need to choose between them. You can say: “Hijama likely works through multiple pathways. The classical scholars described it as removing harmful substances from the blood. Modern research shows it also increases blood flow, modulates the immune system, releases fascia, and stimulates lymphatic drainage. All of these may be happening at once.” That is an honest, complete answer.

Student Notes:

Five mechanisms. Know them all. Communicate them as concurrent, not competing. The Taibah Theory is classical; the others are modern. Both are valid.

Evidence Reference:

Lüdtke et al. (2006); Al-Jauziyah et al. (2015); PMC7735689 (MFD superiority over foam rolling).

SLIDE 019 — Physiological Effects

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 6 minutes

On-Slide Text:

- Title: “PHYSIOLOGICAL EFFECTS BY BODY SYSTEM”
- Body: “Hijama produces measurable physiological effects across multiple body systems. These effects are supported by varying levels of evidence and should be communicated to patients with appropriate qualification.”
- Table (3 columns × 7 rows):
 - Header: System | Effect | Evidence Summary
 - Row 1: Cardiovascular | Reduces SBP by 8.4 mmHg at 4 weeks; improves microcirculation; favourable lipid profile changes | Moderate — PMID: 26559364
 - Row 2: Metabolic | Potential fasting blood glucose reduction in Type 2 diabetes; supports weight management adjunctively | Preliminary — Mahdavi et al., 2015
 - Row 3: Musculoskeletal | MFD increases hamstring flexibility superior to foam rolling; effective for myofascial pain and sports injuries | Moderate — PMC7735689
 - Row 4: Neurological | Modulates pain gate mechanisms; enhances neurotransmission; promotes endorphin release; reduces migraine frequency | Moderate — multiple RCTs
 - Row 5: Immunological | Stimulates lymphatic flow and white blood cell activity; enhances immune surveillance; supports natural defence mechanisms | Moderate — mechanistic studies
 - Row 6: Homeostatic | Supports self-regulatory capacity; enhances nutrient absorption and waste elimination; sensitises skin receptors for improved organ communication | Traditional framework; preliminary modern evidence

Speaker Notes:

This table is your patient communication tool. When someone asks what Hijama can do for their specific condition, you can point to the relevant system and evidence level. But always qualify: “The evidence for cardiovascular effects is moderate. The evidence for metabolic effects is preliminary.” That qualification is not weakness. It is the mark of a practitioner who respects the limits of knowledge — which is itself a prophetic quality.

Student Notes:

Use this table for patient education. Always state the evidence level. Never overstate.

SLIDE 020 — Evidence Evolution Quote

Master: QUOTE_FULL_EMERALD

Background: Deep Emerald full bleed

Duration: 2 minutes

On-Slide Text:

- Large quotation mark: “” — Libre Baskerville, 72 pt, Antique Gold

- English: “The evidence base evolves. Your knowledge must evolve with it. A practitioner who relies on a 2010 understanding in 2026 is not traditional. They are obsolete.” — Libre Baskerville, 16 pt, italic, Warm Ivory
- Source: “— Sunnah Remedies Academy Clinical Manual” — Inter, 10 pt, Antique Gold
- Context: “The Academy’s position is reviewed annually. Updates are published in the Alumni Portal and incorporated into the next revision of the Clinical Manual. Practitioners who graduated under previous versions are expected to familiarise themselves with updates through continuing professional development.” — Inter, 10 pt, Antique Gold

Speaker Notes:

This is your professional obligation for life. CPD is not a box to tick. It is the continuation of the prophetic command to seek knowledge. The evidence in 2026 will be different from 2030. Your practice must evolve. The Academy will support that evolution through the Alumni Portal, annual reviews, and advanced modules.

Student Notes:

Minimum 20 hours CPD per year. Check the Alumni Portal quarterly for evidence updates.

SLIDE 021 — Module 3 Divider

Master: MODULE_DIVIDER_EMERALD

Background: Deep Emerald full bleed

Duration: 1 minute

On-Slide Text:

- Large number: “03” — Libre Baskerville, 120 pt, bold, Antique Gold
- Title: “HUMAN ANATOMY FOR PRACTITIONERS” — Libre Baskerville, 36 pt, bold, Warm Ivory
- Description: “Essential anatomical knowledge for safe and effective practice” — Inter, 14 pt, Antique Gold
- English: “And when I am ill, it is He who cures me.” — Libre Baskerville, 12 pt, italic, Warm Ivory
- Source: “— Surah Ash-Shu’ara, 26:80” — Inter, 9 pt, Antique Gold

Speaker Notes:

Module 3 is where technicians become healers. You cannot practice Hijama safely without knowing what lies beneath the skin. The patient who trusts you with their body deserves a practitioner who knows anatomy better than they know their own name. This module is not optional. It is the difference between healing and harming.

Student Notes:

Module 3 = anatomy. This is the safety module. Do not rush it.

SLIDE 022 — Module 3 Objectives

Master: OBJECTIVES_BULLET

Background: Warm Ivory

Duration: 2 minutes

On-Slide Text:

- Title: “MODULE OBJECTIVES”
- Five bullets:
 1. Describe the three layers of skin relevant to cupping and identify the target depth for scarification
 2. Explain the structure and function of arteries, veins, capillaries, and lymphatic vessels
 3. Locate the major Sunnah points using anatomical landmarks and palpation techniques
 4. Identify muscles, fascia, and neurovascular structures in the back, neck, and shoulder regions
 5. Recognise anatomical contraindications and danger zones for cup placement

Speaker Notes:

Objective 1 is the most critical: 0.5–1 mm into the papillary dermis. That is the difference between a therapeutic extraction and a permanent scar. Objective 5 is the lifesaver: knowing where NOT to place a cup is as important as knowing where to place it.

Student Notes:

Scarification depth: 0.5–1 mm. Danger zones: vertebral artery, carotid sheath, brachial plexus, lymph nodes.

SLIDE 023 — Skin Anatomy

Master: CONTENT_TABLE_4COL

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “SKIN ANATOMY: LAYERS RELEVANT TO CUPPING”
- Body: “The skin is the practitioner’s primary interface with the patient. Understanding its layered structure is not academic — it is the difference between a safe scarification and a permanent injury.”
- Table (4 columns × 4 rows):

Layer	Depth	Structure	Clinical Relevance
Row 1: Epidermis	0.05–1.5 mm	Avascular stratified squamous epithelium; stratum corneum barrier	Provides surface integrity; must be penetrated to access dermal capillaries
Row 2: Dermis	1–4 mm	Papillary layer (capillary loops) and reticular layer (structural support)	PRIMARY target layer; papillary dermis contains superficial capillary networks accessed during scarification
- Header: Layer | Depth | Structure | Clinical Relevance
- Row 1: Epidermis | 0.05–1.5 mm | Avascular stratified squamous epithelium; stratum corneum barrier | Provides surface integrity; must be penetrated to access dermal capillaries
- Row 2: Dermis | 1–4 mm | Papillary layer (capillary loops) and reticular layer (structural support) | PRIMARY target layer; papillary dermis contains superficial capillary networks accessed during scarification

- Row 3: Hypodermis (Subcutaneous) | Variable | Adipose and connective tissue containing larger vessels and nerves | Provides insulation and cushioning; deeper vessels must be avoided during scarification
- Warning box: “SCARIFICATION TARGET DEPTH: 0.5–1 mm into the papillary dermis. Never penetrate the hypodermis. Too shallow: inadequate blood flow. Too deep: risk of scarring, infection, damage to deeper structures.”

Speaker Notes:

Look at the numbers. The epidermis is 0.05–1.5 mm. The dermis is 1–4 mm. Your target is 0.5–1 mm. That means you are barely penetrating the dermis. You are not cutting deeply. You are making a superficial incision into the papillary layer where the capillary loops sit. If you feel resistance, you are too deep. If you see no blood, you are too shallow. The ideal incision releases a small, steady flow of dark blood. That is your feedback loop.

Student Notes:

Target: papillary dermis (0.5–1 mm). Hypodermis = danger zone. Ideal feedback: small steady flow of dark blood.

Warning:

Scarification target depth: 0.5–1 mm into the papillary dermis. Never penetrate the hypodermis. Too shallow: inadequate blood flow. Too deep: risk of scarring, infection, damage to deeper structures.

SLIDE 024 — Skin Thickness Variation

Master: CONTENT_TABLE_4COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “SKIN THICKNESS VARIATION BY BODY SITE”
- Body: “Skin thickness varies dramatically across body sites. A scarification depth appropriate for the back may be dangerously deep for the neck or dangerously shallow for the palms.”
- Table (4 columns × 6 rows):
 - Header: Body Site | Thickness | Scarification Adjustment | Risk Assessment
 - Row 1: Eyelids | 0.5 mm | Never cup or scarify | Extreme risk; full-thickness penetration possible
 - Row 2: Back (Al-Kahil) | 2.0–4.0 mm | Standard depth (0.5–1 mm) | Standard protocol applies
 - Row 3: Abdomen | 1.5–2.5 mm | Slightly reduced depth | Beware of underlying viscera in thin patients
 - Row 4: Palms / Soles | 2.5–5.0 mm | Not standard cupping sites | Thick stratum corneum; limited vascular access
 - Row 5: Neck (Akhda’een) | 1.2–2.0 mm | Reduced depth; extreme caution | Vertebral artery and carotid sheath proximity

Speaker Notes:

The neck is your highest-risk standard site. At 1.2–2.0 mm, the skin is thin. Beneath it lies the stern-

ocleidomastoid, the carotid sheath, and the vertebral artery. A 1 mm incision on the back is safe. A 1 mm incision on the neck is at the limit of safety. Reduce your depth. Reduce your confidence. Increase your caution. The Prophet received Hijama on the neck, but he had the best cupper in his community — and even then, he taught us caution.

Student Notes:

Neck = highest risk standard site. Reduce depth. Increase caution. Never cup over the carotid pulse.

SLIDE 025 — Circulatory System

Master: CONTENT_TABLE_4COL

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “THE CIRCULATORY SYSTEM: BLOOD VESSELS”
- Body: “Every cup placement is a statement of vascular knowledge. The practitioner must know which vessels to avoid, which to access, and which to monitor.”
- Table (4 columns × 6 rows):

Vessel Type	Structure	Function	Clinical Relevance
Arteries	Thick wall, elastic, high pressure	Carry oxygenated blood away from heart	Never cup directly over; palpate before placement
Arterioles	Smaller, more smooth muscle	Regional blood flow control	Respond to cupping vasodilation; contribute to reactive hyperemia
Capillaries	Single endothelial cell layer	Gas and nutrient exchange	PRIMARY vessels accessed during Hijama; target of scarification
Venules	Thin walls, minimal muscle	Collect blood from capillaries	Drain cupping-extracted blood; avoid direct cupping over large venules
Veins	Valves, low pressure, thin wall	Return blood to the heart	Avoid varicose veins; note valve locations for flow patterns
- Header: Vessel Type | Structure | Function | Clinical Relevance
- Row 1: Arteries | Thick wall, elastic, high pressure | Carry oxygenated blood away from heart | Never cup directly over; palpate before placement
- Row 2: Arterioles | Smaller, more smooth muscle | Regional blood flow control | Respond to cupping vasodilation; contribute to reactive hyperemia
- Row 3: Capillaries | Single endothelial cell layer | Gas and nutrient exchange | PRIMARY vessels accessed during Hijama; target of scarification
- Row 4: Venules | Thin walls, minimal muscle | Collect blood from capillaries | Drain cupping-extracted blood; avoid direct cupping over large venules
- Row 5: Veins | Valves, low pressure, thin wall | Return blood to the heart | Avoid varicose veins; note valve locations for flow patterns

Speaker Notes:

The capillary is your target. It is a single cell thick. That is why a 0.5–1 mm incision is sufficient — you are accessing the superficial capillary network in the papillary dermis. Arteries are your danger. If you see bright red blood spurting, you have hit an arteriole. Apply immediate pressure. Document. Refer. Never attempt to continue the procedure. Arterial bleeding is an emergency.

Student Notes:

Capillary = target. Artery = danger. Bright red spurting blood = arterial hit. Pressure. Document. Refer.

SLIDE 026 — Heart Structure

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “THE HEART: STRUCTURE AND FUNCTION”
- Body: “Understanding cardiac function is essential for recognising contraindications, interpreting vital signs, and monitoring patient safety during and after Hijama sessions.”
- Table (3 columns × 5 rows):
- Header: Chamber | Function | Clinical Relevance
- Row 1: Right Atrium | Receives deoxygenated blood via vena cava | Understanding venous return helps interpret post-cupping fatigue
- Row 2: Right Ventricle | Pumps deoxygenated blood to lungs via pulmonary artery | Pulmonary circulation relevance for respiratory patients
- Row 3: Left Atrium | Receives oxygenated blood via pulmonary veins | Pressure monitoring in cardiac patients
- Row 4: Left Ventricle | Thickest wall; pumps to systemic circulation via aorta | Systemic blood pressure generation; key for hypertension management
- Clinical Pearl: “Normal BP: 120/80 mmHg (systolic/diastolic). Pre- and post-treatment BP monitoring is mandatory for all patients, especially those with hypertension or cardiovascular risk factors.”

Speaker Notes:

The left ventricle is your key chamber for hypertension management. It generates systemic pressure. When you reduce blood volume through Hijama, you are reducing preload. In a healthy patient, the body compensates. In a hypertensive patient, the compensation may be delayed. That is why BP monitoring is mandatory. A drop of 20 mmHg post-treatment is normal. A drop of 40 mmHg requires observation. A drop of 60 mmHg requires intervention.

Student Notes:

Normal BP = 120/80. Pre- and post-treatment BP mandatory. Drops >40 mmHg require extended observation.

Clinical Pearl:

Normal BP: 120/80 mmHg (systolic/diastolic). Pre- and post-treatment BP monitoring is mandatory for all patients, especially those with hypertension or cardiovascular risk factors.

SLIDE 027 — Lymphatic System

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “THE LYMPHATIC SYSTEM”
- Body: “The lymphatic system is the body’s drainage and immune surveillance network. Hijama’s negative pressure enhances lymphatic clearance, but improper cup placement can damage lymphatic structures.”
- Table (3 columns × 4 rows):
- Header: Component | Structure & Function | Hijama Relevance
- Row 1: Lymphatic Capillaries | Blind-ended, highly permeable vessels collecting interstitial fluid | Enhanced drainage during cupping; explains oedema benefits
- Row 2: Lymph Nodes | Filter lymph and house immune cells (lymphocytes, macrophages) | Never apply cups directly over lymph nodes; major clusters in neck, axilla, groin
- Row 3: Lymphatic Organs | Spleen, thymus, tonsils — immune cell production and pathogen filtration | Systemic immune modulation supported by lymphatic stimulation
- Warning box: “Never apply cups directly over lymph nodes or major lymphatic ducts. Avoid the axillary, inguinal, and cervical lymph node regions. Damage to lymphatic structures can cause chronic oedema and immune compromise.”

Speaker Notes:

The axilla is a common mistake. Students want to cup the armpit for detoxification. Do not. The axillary lymph nodes are dense and superficial. Cupping over them can cause lymphatic damage that leads to chronic arm oedema. The same applies to the groin and the deep cervical chain. Know your lymph node anatomy. Mark them on your patient before you place a single cup.

Student Notes:

Never cup over: axilla, groin, deep cervical chain. Mark lymph nodes before cup placement.

Warning:

Never apply cups directly over lymph nodes or major lymphatic ducts. Avoid the axillary, inguinal, and cervical lymph node regions. Damage to lymphatic structures can cause chronic oedema and immune compromise.

SLIDE 028 — Fascia & Musculoskeletal

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “FASCIA AND THE MUSCULOSKELETAL SYSTEM”
- Body: “Fascia is a continuous connective tissue network surrounding muscles, organs, and structures. Myofascial restrictions — adhesions within fascial layers — contribute to pain, reduced range of motion, and impaired circulation.”
- Table (3 columns × 4 rows):
- Header: Fascia Type | Location | Hijama Relevance

- Row 1: Superficial Fascia | Subcutaneous layer containing adipose tissue | Primary layer affected by cupping suction; contains superficial vessels
- Row 2: Deep Fascia | Dense regular connective tissue surrounding muscles and muscle groups | Key target for Myofascial Decompression (MFD); inter-layer separation restores gliding
- Row 3: Visceral Fascia | Surrounds organs and suspends them within body cavities | Not directly treated by Hijama; important for anatomical reference
- Clinical Pearl: “Research (PMC7735689) demonstrates MFD’s superiority over foam rolling for hamstring flexibility improvement. Negative pressure creates inter-layer separation in fascia, restoring fascial gliding and reducing restrictions.”

Speaker Notes:

Myofascial Decompression is the modern explanation for what the classical scholars observed: Hijama relieves pain and restores movement. The mechanism is mechanical separation of fascial layers. When you apply negative pressure, you are not just pulling blood to the surface. You are creating space between tissues that have become stuck. That is why patients often report immediate relief and increased range of motion after a single session.

Student Notes:

MFD = mechanical separation of fascial layers. Explains immediate pain relief and increased ROM. Reference: PMC7735689.

Clinical Pearl:

Research (PMC7735689) demonstrates MFD’s superiority over foam rolling for hamstring flexibility improvement. Negative pressure creates inter-layer separation in fascia, restoring fascial gliding and reducing restrictions.

SLIDE 029 — Key Muscles

Master: CONTENT_TABLE_4COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “KEY MUSCLES FOR CUPPING PRACTICE”
- Body: “Understanding muscle attachments, fibre directions, and trigger points enables precise cup placement for musculoskeletal conditions. Always map muscles before placing cups.”
- Table (4 columns × 7 rows):
 - Header: Muscle | Location | Function | Cupping Application
 - Row 1: Trapezius | Upper back and neck | Scapular elevation, rotation, retraction | Al-Kahil, Al-Katifain; tension headache, stress
 - Row 2: Erector Spinae | Paravertebral column | Spinal extension and posture maintenance | Paravertebral points; lower back pain
 - Row 3: Latissimus Dorsi | Lower back to humerus | Shoulder extension, adduction, internal rotation | Posterior torso cupping; shoulder conditions

- Row 4: Rhomboids | Between scapulae | Scapular retraction and stabilisation | Interscapular points; postural correction
- Row 5: Gluteal Groups | Buttock region | Hip extension, abduction, external rotation | Lower back and hip referral patterns
- Row 6: Deltoids | Shoulder cap | Arm abduction, flexion, extension | Shoulder pain, rotator cuff conditions

Speaker Notes:

These six muscles are your primary targets for musculoskeletal cupping. Know their fibre directions — cups should be placed parallel to muscle fibres when possible, perpendicular when treating trigger points. The trapezius is your most common target because it carries stress, tension, and postural dysfunction. The erector spinae is your second most common target for lower back pain. Map these muscles on your hands before you touch a patient.

Student Notes:

Six primary muscles. Know fibre directions. Trapezius = most common. Erector spinae = second most common.

SLIDE 030 — Module 4 Divider

Master: MODULE_DIVIDER_EMERALD

Background: Deep Emerald full bleed

Duration: 1 minute

On-Slide Text:

- Large number: “04” — Libre Baskerville, 120 pt, bold, Antique Gold
- Title: “CLINICAL PRACTICE & SAFETY” — Libre Baskerville, 36 pt, bold, Warm Ivory
- Description: “Equipment, infection control, patient assessment, and risk management” — Inter, 14 pt, Antique Gold
- Arabic: “ ” — Libre Baskerville, 14 pt, italic, Warm Ivory
- English: “There should be no harm nor reciprocal harm.” — Libre Baskerville, 12 pt, italic, Warm Ivory
- Source: “— Hadith, authenticated by Ibn Majah” — Inter, 9 pt, Antique Gold

Speaker Notes:

This hadith is the foundation of everything we do. It is not a decoration. It is a command. Every clinical decision you make must pass this test: does it risk harm? If yes, do not do it. Module 4 is your safety fortress. It is not the most exciting module. It is the most important module. A practitioner who masters safety can practice for forty years. A practitioner who neglects safety will harm someone in their first year.

Student Notes:

Module 4 = safety. Boring but essential. Master this or harm someone.

Islamic Insight:

There should be no harm nor reciprocal harm. — Ibn Majah. This hadith governs every clinical decision.

SLIDE 031 — Module 4 Objectives

Master: OBJECTIVES_BULLET

Background: Warm Ivory

Duration: 2 minutes

On-Slide Text:

- Title: “MODULE OBJECTIVES”
- Five bullets:
 1. Select, maintain, and sterilise equipment according to the Academy Sterility Standard
 2. Implement the WHO 5 Moments of Hand Hygiene and universal precautions
 3. Conduct comprehensive pre-treatment patient assessments including medical history and vital signs
 4. Screen for absolute and relative contraindications using the Risk Stratification Framework
 5. Obtain valid informed consent and maintain clinical documentation to Academy standards

Speaker Notes:

These five objectives are your legal and ethical shield. If you can demonstrate competency in all five, you are protected. If you cannot, you are exposed. The Academy’s insurance, reputation, and mission depend on every graduate meeting these standards.

Student Notes:

Five objectives = legal and ethical shield. All five must be demonstrated in OSCE assessment.

SLIDE 032 — Equipment

Master: CONTENT_TABLE_4COL

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “EQUIPMENT: CUPS, PUMPS, AND INSTRUMENTS”
- Body: “The practitioner’s equipment is an extension of their clinical judgment. Every item must meet medical device regulations, be maintained according to manufacturer specifications, and be inspected before each patient contact.”
- Table (4 columns × 7 rows):

Category	Item	Specification	Sterility Requirement
Row 1:	Cutting	Sterile lancets	Single-use, 21–23 gauge, sterile blade devices Single-use only; never reprocess
Row 2:	Protection	Medical gloves	Nitrile, powder-free, non-latex Change between patients; sterile gloves for wet cupping

- Row 3: Antiseptic | Skin preparation | Chlorhexidine 2% or povidone-iodine | Check expiry; allow adequate contact time
- Row 4: Dressing | Sterile gauze and tape | Non-adherent pads, hypoallergenic tape | Single-use; check integrity before application
- Row 5: Waste | Sharps container | Biohazard, puncture-resistant, UN-approved | Never overfill; seal and dispose via licensed contractor
- Row 6: Waste | Clinical waste bags | Yellow tiger-stripe bags for blood-soiled materials | Segregate at point of use

Speaker Notes:
Every item on this table is non-negotiable. You do not practice without sterile lancets. You do not practice without nitrile gloves. You do not practice without proper waste disposal. The Academy will inspect your clinic before certification. If you are missing any item, you will not pass. This is not bureaucracy. This is patient protection.

Student Notes:
All items non-negotiable. Academy clinic inspection before certification. Missing items = failure.

SLIDE 033 — Cup Types

Master: CONTENT_TABLE_5COL
Background: Warm Ivory
Duration: 4 minutes

- On-Slide Text:**
- Title: “CUP TYPES AND CLINICAL SELECTION”
 - Table (5 columns × 6 rows):
 - Header: Cup Type | Material | Mechanism | Indications | Sterilisation
 - Row 1: Plastic Disposable | Medical-grade polymer | Hand pump valve system | Wet cupping — primary recommendation | Single-use only
 - Row 2: Glass Cups | Borosilicate glass | Fire or pump vacuum | Dry cupping; fire cupping (advanced) | Autoclave 121°C 15 min
 - Row 3: Silicone Cups | Food-grade silicone | Squeeze self-suction | Massage cupping; self-application | High-level disinfection
 - Row 4: Bamboo Cups | Natural bamboo | Fire or pump vacuum | Traditional Chinese applications | Thorough sterilisation between uses

Speaker Notes:
The Academy’s primary recommendation is disposable plastic valve cups for all wet cupping. This eliminates cross-contamination risk, provides consistent suction control, and meets CE/UKCA/FDA medical device regulations. Glass cups are appropriate for dry cupping in autoclave-equipped clinics. Fire cupping is not taught at CHP level. Silicone cups are useful for massage cupping but not for wet cupping due to cleaning challenges.

Student Notes:

Academy recommendation: disposable plastic valve cups for wet cupping. Glass = dry cupping only. Fire cupping = advanced module.

SLIDE 034 — Infection Prevention

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 6 minutes

On-Slide Text:

- Title: “INFECTION PREVENTION AND CONTROL”
- Body: “Infection control is not a checklist to be completed. It is a culture to be embodied. The six pillars protect every patient, every time.”
- Table (3 columns × 7 rows):
- Header: Pillar | Standard | Frequency / Timing
- Row 1: Hand Hygiene | WHO 6-step technique, minimum 40 seconds with soap and water, or 20 seconds with alcohol-based hand rub | Before and after every patient contact; before aseptic procedure; after body fluid exposure
- Row 2: PPE | Gloves minimum; sterile gloves for wet cupping; apron and eye protection when indicated | Donned before patient contact; removed and discarded after
- Row 3: Sterile Technique | Critical items single-use; semi-critical items high-level disinfection or sterilisation | Before every procedure; between patients
- Row 4: Environmental Cleaning | Treatment surface: intermediate-level disinfectant; floors: daily low-level disinfection | Between patients; daily; weekly deep clean
- Row 5: Waste Management | Segregated, labelled, licensed disposal; sharps in rigid containers | At point of generation; never leave unattended
- Row 6: Documentation | Record all infection control measures in patient record and clinic log | Contemporaneous with each procedure
- Clinical Pearl: “‘Cleanliness is half of faith.’ (Sahih Muslim) The hand hygiene station is the most important piece of equipment in your clinic. More important than the pump. More important than the cups. Because clean hands protect every patient, every time.”

Speaker Notes:

The WHO 6-step hand hygiene technique takes 40 seconds. Most people spend 10 seconds. Those 30 seconds of laziness can kill a patient. MRSA, hepatitis B, hepatitis C, and HIV are all transmissible through poor hand hygiene. You are not washing your hands for yourself. You are washing them for the next patient. And the next. And the next. That is amanah.

Student Notes:

WHO 6-step = 40 seconds minimum. MRSA, HBV, HCV, HIV = all preventable with proper hand hygiene.

Clinical Pearl:

‘Cleanliness is half of faith.’ (Sahih Muslim) The hand hygiene station is the most important piece of equipment in your clinic. More important than the pump. More important than the cups. Because clean hands protect every patient, every time.

SLIDE 035 — Sterilisation Hierarchy

Master: CONTENT_TABLE_4COL

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “STERILISATION HIERARCHY BY ITEM CATEGORY”
- Body: “Items are classified by their contact risk. Critical items enter the bloodstream and demand the highest sterility standards. Semi-critical items contact mucosa or non-intact skin. Non-critical items contact intact skin only.”
- Table (4 columns × 6 rows):
- Header: Item Category | Classification | Required Process | Frequency
- Row 1: Lancets, needles | Critical | Single-use disposable; never reprocess | Per patient
- Row 2: Cups (glass/metal) | Semi-critical | High-level disinfection or sterilisation (autoclave) | After each use
- Row 3: Pump, tubing | Semi-critical | High-level disinfection per manufacturer instructions | After each use
- Row 4: Treatment surface | Non-critical | Intermediate-level disinfection | Between patients
- Row 5: Environmental surfaces | Non-critical | Low-level disinfection | Daily

Speaker Notes:

The Spaulding classification is your legal framework. Critical items must be sterile. Lancets are critical. Never reuse a lancet. Never. I will say it again: never reuse a lancet. If you are tempted to reuse a lancet to save money, you are in the wrong profession. The cost of a lancet is 5 pence. The cost of a hepatitis B transmission is a lifetime of illness and a lawsuit that destroys your practice.

Student Notes:

NEVER reuse lancets. Cost of lancet = 5p. Cost of hepatitis B transmission = lifetime illness + lawsuit + decertification.

SLIDE 036 — 7-Step Patient Assessment

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 8 minutes

On-Slide Text:

- Title: “THE 7-STEP PATIENT ASSESSMENT”

- Body: “Every patient must be assessed using the 7-Step framework before any treatment is planned. No shortcuts. No assumptions. The practitioner who skips steps to save time is not efficient. They are negligent.”
- Table (3 columns × 8 rows):
- Header: Step | Component | Critical Elements
- Row 1: 1 | Presenting Complaint and Goals | Record patient’s own words; identify primary concerns and desired outcomes; do not reinterpret or diagnose
- Row 2: 2 | Medical History (Systematic Review) | Cardiovascular, respiratory, GI, GU, musculoskeletal, neurological, endocrine, immunological, psychiatric, dermatological
- Row 3: 3 | Current Medications and Allergies | All prescription, OTC, supplements, herbal remedies; note anticoagulants, antihypertensives, diabetes medications
- Row 4: 4 | Previous Hijama Experience | Dates, practitioners, points used, responses, adverse events; informs approach but does not preclude treatment
- Row 5: 5 | Lifestyle Factors | Diet, hydration, sleep, exercise, stress, occupation, spiritual practice — context for treatment success or failure
- Row 6: 6 | Vital Signs | Blood pressure, heart rate, temperature, oxygen saturation (if indicated), blood glucose (for diabetic patients)
- Row 7: 7 | Risk Stratification and Contraindication Screening | Complete standardised Contraindication Checklist; assign Green, Amber, or Red category
- Warning box: “NEVER SKIP STEP 7. The 7-Step Assessment is not a formality. It is a safety net. Variation introduces error. Systematic practice is safe practice.”

Speaker Notes:

Step 7 is where most mistakes happen. A practitioner who completes steps 1–6 but skips step 7 is like a pilot who checks the engine, the fuel, the weather, but never checks the landing gear. Step 7 is your landing gear. It is where you discover the contraindication that would have killed your patient. The Contraindication Checklist is not optional. It is your legal defence. If you treat a patient with a contraindication and you did not document the checklist, you are negligent. Period.

Student Notes:

Step 7 = landing gear. Never skip. Contraindication Checklist = legal defence. No documentation = negligence.

Warning:

NEVER SKIP STEP 7. The 7-Step Assessment is not a formality. It is a safety net. Variation introduces error. Systematic practice is safe practice.

SLIDE 037 — Red Flags

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 6 minutes

On-Slide Text:

- Title: “RED FLAGS REQUIRING MEDICAL REFERRAL”
- Body: “The Red Line exists to protect the patient. A practitioner who never refers is a practitioner who eventually harms. A practitioner who refers appropriately embodies the Islamic principle of amanah.”
- Table (3 columns × 8 rows):
- Header: System | Red Flag | Action
- Row 1: Cardiovascular | Chest pain, dyspnoea, palpitations with syncope, BP >180/110 | Emergency referral; do not treat
- Row 2: Neurological | New weakness, numbness, speech disturbance, severe headache, altered consciousness | Emergency referral
- Row 3: Oncological | Unexplained weight loss, night sweats, persistent lymphadenopathy, new masses | Urgent medical assessment; do not treat
- Row 4: Infectious | Fever with rigors, unexplained fever >38°C, signs of sepsis | Emergency referral
- Row 5: Musculoskeletal | Hot, swollen joint with systemic symptoms; suspected fracture | Medical assessment; do not treat
- Row 6: Dermatological | Rapidly changing moles, non-healing ulcers, signs of cellulitis | Medical assessment
- Row 7: Psychiatric | Acute psychosis, suicidal ideation, severe agitation | Emergency mental health referral

Speaker Notes:

When in doubt, refer. It is better to over-refer than to under-treat. The patient with chest pain might have indigestion. But it might be a myocardial infarction. You are not qualified to distinguish between the two. That is not your job. Your job is to recognise that chest pain is a red flag and refer immediately. The Prophet said: “Whoever believes in Allah and the Last Day, let him speak good or remain silent.” When you do not know, remain silent. When you are uncertain, refer. That is wisdom.

Student Notes:

When in doubt, refer. Better to over-refer than under-treat. Chest pain = MI until proven otherwise. Not your job to diagnose.

Islamic Insight:

Whoever believes in Allah and the Last Day, let him speak good or remain silent. — Sahih al-Bukhari. When you do not know, remain silent. When you are uncertain, refer. This is not silence of ignorance. It is silence of wisdom.

SLIDE 038 — Absolute Contraindications

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “ABSOLUTE CONTRAINDICATIONS”

- Body: “Absolute means absolute. The phrase ‘I’ve done it before and it was fine’ is not a clinical justification. The phrase ‘The patient insisted’ is not a legal defence.”
- Table (3 columns × 9 rows):
- Header: Condition | Rationale | Academy Action
- Row 1: Haemophilia or other bleeding disorders | Risk of uncontrolled bleeding | Do not proceed; document and refer
- Row 2: Severe anaemia (Hb <8 g/dL) without medical supervision | Further blood loss dangerous | Do not proceed; refer for medical management
- Row 3: Active infection at treatment site | Risk of spreading infection; poor healing | Do not proceed; treat infection first
- Row 4: Severe dehydration or shock | Compromised cardiovascular status | Do not proceed; emergency referral
- Row 5: Pregnancy (abdominal/lumbar cupping) | Risk to fetus; uterine stimulation | Absolute contraindication for abdominal/lumbar sites
- Row 6: Uncontrolled hypertension (>180/110 mmHg) | Risk of hypertensive crisis | Do not proceed; urgent medical referral
- Row 7: Suspected deep vein thrombosis (DVT) | Risk of embolism | Do not proceed; emergency referral
- Row 8: Patient refusal | Autonomy is absolute | Do not proceed under any circumstances

Speaker Notes:

These eight conditions are non-negotiable. There are no exceptions. Not for family. Not for friends. Not for wealthy patients who offer to pay extra. The absolute contraindications exist because the risk of harm outweighs any potential benefit. The practitioner who makes exceptions is not exercising clinical judgment — they are exercising clinical arrogance. And arrogance harms patients.

Student Notes:

Eight absolute contraindications. No exceptions. Not for family. Not for money. Not for any reason.

SLIDE 039 — Relative Contraindications

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “RELATIVE CONTRAINDICATIONS”
- Body: “Relative contraindications require individualised risk-benefit assessment, documented clinical reasoning, and often physician consultation. Proceed with caution. Document all modifications.”
- Table (3 columns × 10 rows):
- Header: Condition | Precaution | Modification

- Row 1: Diabetes mellitus | Risk of poor wound healing, infection | Glucose monitoring; reduced scarification depth; enhanced aftercare
- Row 2: Hypertension (controlled) | BP fluctuation | Pre- and post-treatment BP monitoring; avoid Akhda'een if unstable
- Row 3: Epilepsy | Seizure risk during procedure | Safe environment; avoid neck points; short sessions
- Row 4: Immunocompromised state | Infection susceptibility | Enhanced sterility; physician consult
- Row 5: History of keloid scarring | Hypertrophic scarring risk | Avoid high-risk areas; test patch
- Row 6: Active skin condition at site | Koebner phenomenon, infection | Treat skin first; select alternative points
- Row 7: Anticoagulant medication | Bleeding risk | Physician clearance; reduce cups; extended pressure
- Row 8: Age >65 | Skin fragility; comorbidities | Gentle suction; reduced volume; close monitoring
- Row 9: Age 16–18 | Consent complexity; smaller body | Guardian consent; chaperone; reduced parameters

Speaker Notes:

Relative contraindications are where your clinical judgment is tested. A diabetic patient is not automatically excluded. But they require glucose monitoring, reduced scarification depth, and enhanced aftercare. An anticoagulant patient is not automatically excluded. But they require physician clearance, reduced cup numbers, and extended pressure time. The key is: document everything. Your reasoning is your defence.

Student Notes:

Relative = individualised assessment. Document everything. Your reasoning = your legal defence.

SLIDE 040 — Risk Stratification

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “THE RISK STRATIFICATION FRAMEWORK”
- Body: “Every patient is assigned to one of three categories. The category determines the protocol. When uncertain, categorise as Amber. When very uncertain, categorise as Red.”
- Table (3 columns × 4 rows):
- Header: Category | Criteria | Protocol
- Row 1: GREEN — Proceed | No contraindications; vital signs stable; previous positive experience; clear treatment goals | Standard protocol with full parameters
- Row 2: AMBER — Proceed with Caution | Relative contraindication(s) present but managed; first-time patient with anxiety; borderline vital signs; complex medical history but stable | Modified protocol; enhanced monitoring; documented reasoning

- Row 3: RED — Defer or Refer | Any absolute contraindication; unclear diagnosis; patient unable to give informed consent; practitioner uncertainty about safety | Do not proceed; refer to appropriate medical professional
- Clinical Pearl: “Uncertainty is not a weakness. It is the mark of a practitioner who respects the limits of their knowledge. The Prophet said: ‘The believer is not stung from the same hole twice.’ Learn from caution.”

Speaker Notes:

The Risk Stratification Framework is your clinical decision tool. Green means go — but still follow the protocol. Amber means caution — modify, monitor, document. Red means stop — no exceptions. When you are uncertain, categorise as Amber. When you are very uncertain, categorise as Red. Uncertainty is not weakness. It is the mark of a practitioner who respects the limits of their knowledge. And that respect protects patients.

Student Notes:

Green = standard protocol. Amber = modified protocol. Red = stop. Uncertain = Amber. Very uncertain = Red.

Clinical Pearl:

Uncertainty is not a weakness. It is the mark of a practitioner who respects the limits of their knowledge. The Prophet said: ‘The believer is not stung from the same hole twice.’ Learn from caution.

SLIDE 041 — Informed Consent

Master: CONTENT_TABLE_2COL

Background: Warm Ivory

Duration: 6 minutes

On-Slide Text:

- Title: “INFORMED CONSENT: THE SACRED CONVERSATION”
- Body: “Consent is not a form. It is a conversation. The signature on the paper is evidence that the conversation took place. It is not a substitute for the conversation.”
- Table (2 columns × 9 rows):
- Header: Element | Description
- Row 1: 1. Nature of Procedure | Explanation of what Hijama involves, including wet vs. dry cupping, duration, and expected sensations
- Row 2: 2. Purpose / Benefits | Expected outcomes based on evidence and Sunnah foundation, with honest limitation of claims
- Row 3: 3. Risks & Side Effects | Bruising, scarring, infection, dizziness, bleeding — honest disclosure of all risks without minimisation
- Row 4: 4. Alternatives | Other treatments available, including conventional medical options and lifestyle modifications
- Row 5: 5. Practitioner Qualifications | CHP certification status, training background, scope of practice, and professional indemnity

- Row 6: 6. Right to Refuse | Patient may withdraw consent at any time without penalty, even during the procedure
- Row 7: 7. Confidentiality | How records are stored, who has access, data protection compliance, and retention periods
- Row 8: 8. Questions & Clarification | Opportunity for patient to ask questions and receive answers in language they understand
- Islamic Insight box: “Informed consent is the practical application of ‘There should be no harm nor reciprocal harm.’ To treat without informing the patient of risks is to risk harm without their knowledge. To obtain informed consent is to honour their autonomy and their dignity as a creation of Allah.”

Speaker Notes:

This is the most legally important slide in the entire programme. Informed consent is not a signature. It is a conversation. You must explain the procedure, the benefits, the risks, the alternatives, your qualifications, their right to refuse, confidentiality, and answer their questions. If they sign without understanding, they have not consented. They have given a signature. And a signature without understanding is worthless in court. Take your time with this. Never rush consent.

Student Notes:

Consent = conversation, not signature. Eight elements. All mandatory. Document the conversation, not just the signature.

Islamic Insight:

Informed consent is the practical application of ‘There should be no harm nor reciprocal harm.’ To treat without informing the patient of risks is to risk harm without their knowledge. To obtain informed consent is to honour their autonomy and their dignity as a creation of Allah.

SLIDE 042 — Module 5 Divider

Master: MODULE_DIVIDER_EMERALD

Background: Deep Emerald full bleed

Duration: 1 minute

On-Slide Text:

- Large number: “05” — Libre Baskerville, 120 pt, bold, Antique Gold
- Title: “TREATMENT PROTOCOLS” — Libre Baskerville, 36 pt, bold, Warm Ivory
- Description: “Sunnah points, procedure technique, and condition-specific applications” — Inter, 14 pt, Antique Gold
- Arabic: “ ” — Libre Baskerville, 14 pt, italic, Warm Ivory
- English: “The best of remedies you have is cupping (Hijama)...” — Libre Baskerville, 12 pt, italic, Warm Ivory
- Source: “— Sahih al-Bukhari, 5357” — Inter, 9 pt, Antique Gold

Speaker Notes:

Module 5 is where knowledge becomes action. You have learned the history, the evidence, the anatomy,

and the safety protocols. Now you will learn the precise technique that transforms a student into a practitioner. Every movement matters. Every parameter matters. This is not art. This is science applied with reverence.

Student Notes:

Module 5 = technical mastery. Every movement matters. Every parameter matters.

SLIDE 043 — Module 5 Objectives

Master: OBJECTIVES_BULLET

Background: Warm Ivory

Duration: 2 minutes

On-Slide Text:

- Title: “MODULE OBJECTIVES”
- Five bullets:
 1. Locate and palpate all primary and secondary Sunnah points with anatomical precision
 2. Execute the 9-step wet cupping procedure according to the SAFE Hijama Protocol
 3. Apply the Standard Health Detox Protocol with appropriate individualisation
 4. Select condition-specific points based on evidence and clinical reasoning
 5. Modify protocols safely for special populations including diabetics, elderly, and anticoagulant patients

Speaker Notes:

These objectives will be assessed in your OSCE practical examination. You will be expected to demonstrate every step of the 9-step procedure on a simulated patient. You will be expected to locate Sunnah points by palpation without visual reference. You will be expected to explain your modifications for a special population case study. This is not theoretical. This is your hands-on certification.

Student Notes:

All five objectives assessed in OSCE. Palpation must be without visual reference. Be prepared.

SLIDE 044 — Sunnah Points

Master: CONTENT_TABLE_5COL

Background: Warm Ivory

Duration: 6 minutes

On-Slide Text:

- Title: “SUNNAH POINTS: ANATOMICAL LANDMARKS”
- Body: “These points are applied bilaterally where applicable, with the Kahil point considered the most emphasised in prophetic tradition. Always combine Sunnah points with condition-specific points for maximum efficacy. Every point requires anatomical verification by palpation before cup placement.”

- Table (5 columns × 6 rows):
- Header: Point | Location | Prophetic Basis | Clinical Application | Caution
- Row 1: Al-Kahil (PRIMARY) | Between shoulder blades (DU14/DU15), midline, T1–T3 | Primary Sunnah point; most frequently recommended by the Prophet | General wellness, detoxification, back pain, immune support | Avoid over bony prominences
- Row 2: Akhda’een (SECONDARY) | Lateral neck, posterior to sternocleidomastoid | Authentic hadith — two veins at sides of neck | Headache, hypertension, thyroid support (with caution) | Vertebral artery proximity
- Row 3: Al-Hamma | Vertex (top of head), midline | Scholarly recommendation from classical texts | Migraine, neurological conditions (limited evidence) | Apply with caution
- Row 4: Umm al-Muqeeth | Occipital region, base of skull | Traditional practice documented by scholars | Tension headache, cervical pain, mental clarity | Cervical spine awareness
- Row 5: Al-Katifain | Shoulder points (GB21), upper trapezius | Authentic hadith — shoulder cupping | Shoulder pain, myofascial tension, stress | Avoid brachial plexus

Speaker Notes:

This table is your clinical bible. Memorise it. Every point. Every location. Every caution. In your OSCE, you will be asked to palpate Al-Kahil on a model. If you cannot find T1–T3 by palpation, you fail. If you place Akhda’een over the carotid artery, you fail. Anatomical precision is not optional. It is the difference between healing and stroke.

Student Notes:

Memorise this table. OSCE = palpate Al-Kahil without visual reference. Fail = incorrect placement.

Clinical Pearl:

Always combine Sunnah points with condition-specific points for maximum efficacy. The Sunnah is the foundation, not the ceiling.

SLIDE 045 — Standard Health Detox Protocol

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “STANDARD HEALTH DETOX PROTOCOL”
- Body: “The Standard Health Detox Protocol is the foundation of Academy practice. All treatment plans must be individualised based on patient assessment, contraindications screening, and clinical reasoning. Never apply a one-size-fits-all approach.”
- Table (3 columns × 8 rows):
- Header: Parameter | Standard | Clinical Notes
- Row 1: Cup Type | Plastic disposable with valve | Single-use only; various sizes per body site
- Row 2: Number of Cups | 5–7 cups per session | Individualised based on patient condition and tolerance

- Row 3: Suction Pressure | −0.05 to −0.08 MPa | Use pressure gauge; reduce for elderly or sensitive skin
- Row 4: Dry Duration | 5–10 minutes per cup | Monitor skin colour changes throughout
- Row 5: Wet Pre-Incision | 3–5 minutes before scarification | Wait for clear skin elevation and hyperemia
- Row 6: Blood Volume | 50–150 ml total per session | Monitor for colour change: dark to lighter/ruddier
- Row 7: Frequency | Monthly maintenance; weekly acute | Maximum 3–4 consecutive weeks for acute conditions, then review
- Clinical Pearl: “Golden Rule: Place cups on or near the source of pain for maximum efficacy. For systemic conditions, place near the affected organ or use Sunnah points for general detoxification.”

Speaker Notes:

These parameters are your starting point, not your destination. A 25-year-old athlete with chronic back pain can tolerate 7 cups at −0.08 MPa. A 70-year-old with thin skin and comorbidities might need 3 cups at −0.03 MPa. Individualisation is not optional. It is the hallmark of clinical competence. The pressure gauge is your best friend. Never guess pressure. Always measure.

Student Notes:

Starting point, not destination. Individualise every parameter. Pressure gauge = mandatory. Never guess.

Clinical Pearl:

Golden Rule: Place cups on or near the source of pain for maximum efficacy. For systemic conditions, place near the affected organ or use Sunnah points for general detoxification.

SLIDE 046 — 9-Step Wet Cupping Procedure

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 10 minutes

On-Slide Text:

- Title: “WET CUPPING PROCEDURE: STEP-BY-STEP”
- Body: “The 9-step procedure is performed within the SAFE Hijama Protocol: Systematic, Anatomical, Faithful, Evidence-Informed. Memorise this until it becomes muscle memory. When you are tired, when you are rushed, when the clinic is full — the protocol is your protection.”
- Table (3 columns × 10 rows):
- Header: Step | Action | Key Parameters & Checks
- Row 1: 1. Preparation | Verify consent, position patient, drape, clean skin with antiseptic | Consent signed and dated; identity confirmed; skin integrity assessed
- Row 2: 2. Cup Placement | Apply cups to predetermined points using anatomical landmarks | Ensure seal is secure; verify no underlying vessels

- Row 3: 3. Waiting Period | Maintain suction 3–5 min until skin elevation and hyperemia observed | Monitor skin colour; patient comfort check
- Row 4: 4. Removal | Release suction and remove cups carefully | Controlled release; avoid sudden decompression
- Row 5: 5. Scarification | 3–5 superficial parallel incisions (0.5–1 mm depth, 5–7 mm length) | Depth: papillary dermis only; parallel to skin tension lines
- Row 6: 6. Reapplication | Replace cup over incisions with moderate suction (–0.03 to –0.05 MPa) | Monitor patient response; watch for excessive bleeding
- Row 7: 7. Collection | Allow flow until blood changes from dark/thick to lighter/runnier | Monitor volume; do not exceed 150 ml total
- Row 8: 8. Completion | Remove cups, clean with antiseptic, apply sterile dressing | Document points, volume, and observations
- Row 9: 9. Aftercare | Provide written instructions, schedule follow-up, monitor for adverse reactions | Written and verbal instructions; emergency contact provided
- Warning box: “CRITICAL — SCARIFICATION DEPTH: Target 0.5–1 mm into the dermis. Never penetrate the hypodermis. Too shallow: inadequate blood flow. Too deep: risk of scarring, infection, damage to deeper structures.”

Speaker Notes:

This is the core of your certification. Every step must be performed in sequence. No skipping. No shortcuts. Step 5 — scarification — is where most injuries occur. The depth is 0.5–1 mm. That is the thickness of a credit card. If you are pressing hard, you are too deep. The incision should glide through the skin with minimal resistance. If you feel resistance, stop. You are at the reticular dermis. That is too deep. Practice on synthetic skin models until you can consistently achieve the correct depth without thinking.

Student Notes:

Core certification skill. Practice on synthetic models until depth is automatic. 0.5–1 mm = credit card thickness. Resistance = too deep. Stop.

Warning:

CRITICAL — SCARIFICATION DEPTH: Target 0.5–1 mm into the dermis. Never penetrate the hypodermis. Too shallow: inadequate blood flow. Too deep: risk of scarring, infection, damage to deeper structures.

SLIDE 047 — Condition-Specific Applications

Master: CONTENT_TABLE_4COL

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “CONDITION-SPECIFIC APPLICATIONS”

- Body: “The following applications are supported by varying levels of evidence. Practitioners must communicate evidence limitations honestly and never claim Hijama cures any condition definitively. Healing is from Allah alone.”
- Table (4 columns × 8 rows):
- Header: Category | Condition | Evidence Summary | Evidence Grade
- Row 1: Musculoskeletal | Low back pain | 11 RCTs; significant pain and disability improvement | Strong
- Row 2: Musculoskeletal | Neck pain / Myofascial pain | Consistent reduction in pain scores and improved ROM | Moderate
- Row 3: Metabolic | Type 2 Diabetes | Improved fasting glucose; preliminary findings | Preliminary
- Row 4: Metabolic | Dyslipidaemia | Reduced LDL, increased HDL cholesterol | Promising
- Row 5: Cardiovascular | Hypertension | SBP reduced 8.4 mmHg at 4 weeks | Moderate
- Row 6: Reproductive | Female infertility | 20.3% pregnancy rate in pilot study | Preliminary
- Row 7: Neurological | Migraine / Headache | Reduced frequency and intensity | Moderate

Speaker Notes:

This table is your patient communication tool. When someone asks what Hijama can do for their condition, you can point to the evidence grade. But you must always add the qualification. “The evidence for low back pain is strong. The evidence for infertility is preliminary.” That honesty is what separates an Academy graduate from a charlatan. The charlatan promises cures. The graduate promises evidence-informed practice.

Student Notes:

Use for patient communication. Always state evidence grade. Strong = low back pain only. Everything else = moderate or preliminary.

SLIDE 048 — Special Populations

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “MANAGING SPECIAL POPULATIONS”
- Body: “Special populations require modified protocols, enhanced monitoring, and often physician collaboration. The standard protocol is a starting point, not a prescription.”
- Table (3 columns × 7 rows):
- Header: Population | Key Considerations | Protocol Modifications
- Row 1: Diabetic Patients | Pre- and post-treatment glucose monitoring; delayed healing risk | Never treat during fasting or missed meals; extended observation; enhanced wound care
- Row 2: Hypertensive Patients | BP check before treatment; medication compliance | Avoid if >160/100 mmHg; monitor for 30 minutes post-treatment

- Row 3: Elderly Patients (70+) | Thinner skin; longer healing times; comorbidities | Reduced suction and shallower incisions; comprehensive medication review
- Row 4: Children (16+ with consent) | Reduced cup numbers; lighter suction; consent complexity | Parental consent and GP awareness mandatory; extra reassurance
- Row 5: Women's Considerations | Menstruation increases bleeding risk; pregnancy is absolute contraindication | Avoid during menstruation; postpartum: wait until fully recovered
- Row 6: Anticoagulant Patients | Individualised risk assessment; bleeding risk | Single-cup trial with extended observation; document thoroughly; consider GP consultation

Speaker Notes:

Special populations are not obstacles. They are opportunities to demonstrate clinical excellence. A diabetic patient who receives safe, modified Hijama will become your most loyal advocate. An elderly patient who feels cared for, not rushed, will refer their entire family. But these populations also carry the highest risk. One mistake with an anticoagulant patient can cause a haematoma that requires surgical evacuation. Respect the risk. Modify the protocol. Document everything.

Student Notes:

Special populations = highest risk + highest reward. Modify protocols. Document everything. One mistake = disaster.

SLIDE 049 — Module 5 Transition

Master: TRANSITION_SPLIT

Background: Left panel Deep Emerald; right panel Warm Ivory

Duration: 1 minute

On-Slide Text:

- Left panel: "NEXT SECTION" — Inter, 10 pt, Antique Gold
- Left panel: "PROFESSIONAL STANDARDS & OPERATIONS" — Libre Baskerville, 24 pt, bold, Warm Ivory
- Right panel: "With equipment prepared, infection control established, and patient assessment complete, we now turn to the precise techniques of Sunnah point selection, controlled scarification, and evidence-informed treatment delivery." — Inter, 12 pt, Charcoal

Speaker Notes:

We transition now from technique to identity. Module 6 is not about what you do. It is about who you are. A technician can perform the 9-step procedure. A professional lives the ethical framework. That is what Module 6 will teach you.

Student Notes:

Transition to Module 6. From technique to identity. From doing to being.

SLIDE 050 — Module 6 Divider

Master: MODULE_DIVIDER_EMERALD

Background: Deep Emerald full bleed

Duration: 1 minute

On-Slide Text:

- Large number: “06” — Libre Baskerville, 120 pt, bold, Antique Gold
- Title: “PROFESSIONAL STANDARDS & OPERATIONS” — Libre Baskerville, 36 pt, bold, Warm Ivory
- Description: “Complications management, documentation systems, and clinic operations” — Inter, 14 pt, Antique Gold
- Arabic: “
” — Libre Baskerville, 14 pt, italic, Warm Ivory
- English: “Whoever believes in Allah and the Last Day, let him speak good or remain silent.” — Libre Baskerville, 12 pt, italic, Warm Ivory
- Source: “— Sahih al-Bukhari” — Inter, 9 pt, Antique Gold

Speaker Notes:

This hadith is the foundation of professional communication. When you advertise your services, speak good or remain silent. When you discuss outcomes with patients, speak good or remain silent. When you are tempted to exaggerate to attract clients, remember this hadith. The Prophet did not say “speak whatever sells.” He said “speak good or remain silent.” That is your advertising standard. That is your patient communication standard. That is your professional identity.

Student Notes:

Module 6 = professional identity. Speak good or remain silent. This governs advertising, patient communication, and all professional interactions.

Islamic Insight:

Whoever believes in Allah and the Last Day, let him speak good or remain silent. — Sahih al-Bukhari. This governs every word you speak as a practitioner.

SLIDE 051 — Module 6 Objectives

Master: OBJECTIVES_BULLET

Background: Warm Ivory

Duration: 2 minutes

On-Slide Text:

- Title: “MODULE OBJECTIVES”
- Five bullets:
 1. Recognise, classify, and manage adverse events according to Academy protocols
 2. Distinguish between normal Hijama marks and pathological bruising or complications
 3. Establish and maintain a clinic environment meeting Academy safety and hygiene standards
 4. Navigate fasting considerations during Ramadan with patient safety as the primary criterion
 5. Demonstrate the six core competencies required for CHP certification

Speaker Notes:

These objectives complete your professional formation. You will not merely be a cupper. You will be a Certified Hijama Practitioner — a title that carries legal, ethical, and spiritual weight. Treat it with the reverence it deserves.

Student Notes:

Five objectives complete professional formation. CHP = legal, ethical, and spiritual title. Treat it with reverence.

SLIDE 052 — Adverse Events

Master: CONTENT_TABLE_4COL

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “ADVERSE EVENTS: CLASSIFICATION AND MANAGEMENT”
- Body: “All adverse events must be documented, reported to your professional indemnity insurer, and reviewed for learning and quality improvement. Transparency is the foundation of trust.”
- Table (4 columns × 4 rows):
- Header: Classification | Definition | Examples | Action Required
- Row 1: Minor | No intervention required; patient fully recovers | Bruising, mild dizziness, temporary discomfort | Document; monitor; inform patient
- Row 2: Moderate | Requires intervention; patient recovers without lasting harm | Prolonged bleeding, syncope, minor infection | Document; treat; report to Clinical Director
- Row 3: Serious | Requires hospitalisation; risk of lasting harm or death | Severe infection, anaphylaxis, embolism | Document; emergency referral; immediate report to Clinical Director and regulatory authority
- Warning box: “Failure to report is a serious breach. The practitioner who hides an adverse event is not protecting the Academy. They are destroying it. Just culture vs. blame culture: The Academy investigates incidents to learn, not to punish. Honest reporting is protected. Concealment is not.”

Speaker Notes:

Adverse events will happen. Bruising is normal. Dizziness is common. These are minor events — document them, inform the patient, and move on. But if a patient faints, that is moderate. If a patient has anaphylaxis, that is serious. The classification determines your response. Minor = you handle it. Moderate = you handle it and report. Serious = you refer immediately and report within the hour. Never hide an adverse event. The Academy’s just culture protects honest reporters. It does not protect concealers.

Student Notes:

Adverse events will happen. Minor = document. Moderate = document + report. Serious = emergency refer + immediate report. Never hide anything.

Warning:

Failure to report is a serious breach. The practitioner who hides an adverse event is not protecting the

Academy. They are destroying it. Just culture vs. blame culture: The Academy investigates incidents to learn, not to punish. Honest reporting is protected. Concealment is not.

SLIDE 053 — Emergency Response

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 6 minutes

On-Slide Text:

- Title: “EMERGENCY RESPONSE PROTOCOLS”
- Body: “All practitioners must maintain current BLS/CPR certification. Emergency equipment must be checked, maintained, and accessible at all times.”
- Table (3 columns × 6 rows):
- Header: Emergency | Recognition | Immediate Action
- Row 1: Syncope | Pale, clammy, yawning, slow pulse, loss of consciousness | Release cups, lower head, cool compress, monitor. If not recovered in 5 min: call emergency services
- Row 2: Excessive Bleeding | Soaking gauze; bleeding >5 min | Direct pressure 10–15 min; elevate; emergency if continues >10 min
- Row 3: Anaphylaxis | Difficulty breathing, swelling, hives | Epinephrine if trained. Call emergency services immediately
- Row 4: Chest Pain | Crushing pain, radiating to arm/jaw, dyspnoea | Call emergency services immediately. Do not treat.
- Row 5: Seizure | Loss of consciousness, tonic-clonic movements | Protect from injury, do not restrain, time. If >5 min: call emergency services

Speaker Notes:

These five emergencies are your preparedness checklist. Syncope is the most common — usually vasovagal, usually resolves with positioning. But if it lasts more than 5 minutes, it is not simple syncope. Call emergency services. Excessive bleeding is usually controlled with pressure. But if it continues beyond 10 minutes despite pressure, that is a coagulation problem. Call emergency services. Anaphylaxis, chest pain, and seizures are all emergency service calls immediately. Do not hesitate. Do not try to manage them yourself. Call. Document. Support the patient until help arrives.

Student Notes:

Five emergencies. Syncope = most common. Anaphylaxis/chest pain/seizure = call immediately. Never hesitate.

SLIDE 054 — Emergency Equipment

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 3 minutes

On-Slide Text:

- Title: “EMERGENCY EQUIPMENT CHECKLIST”
- Body: “Update emergency contact numbers quarterly. An outdated emergency number is as dangerous as no number at all. Test your emergency response plan with a drill at least twice per year.”
- Table (3 columns × 8 rows):
 - Header: Item | Purpose | Check Frequency
 - Row 1: First aid kit | Wound care, bandages, antiseptic | Weekly
 - Row 2: AED (Automated External Defibrillator) | Cardiac arrest | Monthly
 - Row 3: Epinephrine auto-injector | Anaphylaxis | Monthly (check expiry)
 - Row 4: Oxygen and delivery mask | Hypoxia, respiratory distress | Weekly
 - Row 5: Blood pressure monitor | Vital signs assessment | Daily
 - Row 6: Emergency contact list | Rapid communication | Quarterly (update numbers)
 - Row 7: Fire extinguisher | Fire safety | Monthly

Speaker Notes:

This checklist is your legal and moral obligation. If a patient has anaphylaxis and you do not have epinephrine, you are negligent. If a patient has cardiac arrest and you do not have an AED, you are negligent. These items are not expensive. An AED costs £800. Epinephrine costs £50. Professional indemnity costs £500 per year. The cost of negligence is your career, your reputation, and potentially a patient’s life. Invest in safety.

Student Notes:

All items mandatory. AED = £800. Epinephrine = £50. Negligence = career + reputation + potentially life. Invest in safety.

SLIDE 055 — Marks vs Bruising

Master: TWO_COLUMN

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “HIJAMA MARKS VS. BRUISING”
- Left column title: “BRUISING (TRAUMATIC)” — Inter, 12 pt, bold, Deep Emerald
- Left body: “Results from traumatic capillary rupture Blood extravasation into tissue Causes pain, swelling, and colour changes Red-purple-yellow over time Irregular shape, diffuse edges Associated with tissue damage”
- Right column title: “HIJAMA MARKS (NON-TRAUMATIC)” — Inter, 12 pt, bold, Deep Emerald

- Right body: “Negative-pressure-induced capillary dilation Pooled deoxygenated blood/toxins No internal bleeding, minimal pain Circular shape matching cup diameter Resolves 4–10 days Normal physiological response”
- Vertical gold divider

Speaker Notes:

Patient education is your most powerful tool for reducing anxiety and building trust. Most patients have never seen Hijama marks before. They will be alarmed by the dark purple circles on their back. You must educate them before the treatment. Show them this slide. Explain that marks are normal, expected, and non-traumatic. Explain that the colour does not indicate toxicity level. Explain that resolution takes 4–10 days. If you do not educate them, they will panic. And panicked patients do not return.

Student Notes:

Educate BEFORE treatment. Show this slide. Marks = normal, expected, non-traumatic. Colour toxicity. Resolution = 4–10 days.

SLIDE 056 — Mark Colour Interpretation

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 3 minutes

On-Slide Text:

- Title: “MARK COLOUR INTERPRETATION”
- Body: “Mark colour provides clinical information but must not be overstated. Darker colour does not mean more toxins extracted. Each person heals at different rates.”
- Table (3 columns × 5 rows):
- Header: Colour | Interpretation | Clinical Notes
- Row 1: Light pink | Minimal stagnation; good circulation | Common in healthy, active patients
- Row 2: Purple | Moderate stagnation and toxicity | Most common response; indicates therapeutic effect
- Row 3: Dark purple / black | Significant stagnation | Colour does not correlate with blood volume extracted
- Row 4: Red with petechiae | Superficial capillary response | Normal in sensitive skin or lighter suction
- Clinical Pearl: “Patient Education: Marks are normal and expected. Darker colour does not mean more toxins extracted. Each person heals at different rates. Itchiness during healing is normal. Keep area clean and dry.”

Speaker Notes:

Do not overinterpret mark colour. A patient with dark purple marks is not “more toxic” than a patient with light pink marks. Colour is influenced by skin type, suction pressure, duration, and individual vascular response. Use colour as one data point among many. Never use it to diagnose. Never use it to frighten or reassure patients inappropriately.

Student Notes:

Colour = one data point. Not diagnostic. Not indicative of toxicity level. Never overinterpret.

Clinical Pearl:

Patient Education: Marks are normal and expected. Darker colour does not mean more toxins extracted. Each person heals at different rates. Itchiness during healing is normal. Keep area clean and dry.

SLIDE 057 — Clinic Operations

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 5 minutes

On-Slide Text:

- Title: “CLINIC OPERATIONS AND BUSINESS STANDARDS”
- Body: “The clinic environment is an extension of professional identity. It must communicate competence, cleanliness, and care before a single word is spoken.”
- Table (3 columns × 7 rows):
 - Header: Requirement | Standard | Rationale
 - Row 1: Treatment Room | Dedicated space with adequate ventilation and lighting; clinical sink | Infection control; patient dignity; practitioner safety
 - Row 2: Storage | Appropriate storage for sterile and non-sterile items; locked medication cabinet if applicable | Prevent contamination; regulatory compliance
 - Row 3: Waste Storage | Clinical waste storage area; segregated at point of generation | Infection control; licensed disposal requirements
 - Row 4: Emergency Equipment | First aid kit, AED recommended, emergency contacts displayed | Preparedness; duty of care; professional indemnity requirements
 - Row 5: Insurance | Professional indemnity (min £1M); public liability; employer’s liability if applicable | Legal requirement; patient protection; practitioner protection
 - Row 6: CPD | Minimum 20 hours continuing professional development per year | Competency maintenance; evidence updates; professional growth

Speaker Notes:

Your clinic is your brand. When a patient walks in, they should immediately feel: “This is a professional medical environment.” Not a spa. Not a living room. A clinic. White walls, clean surfaces, organised equipment, proper lighting, ventilation, and hand hygiene stations. The Academy will inspect your clinic before certification. If it does not meet standard, you will not pass. This is not punishment. This is protection — for you and your patients.

Student Notes:

Clinic = brand. Must feel professional medical environment. Academy inspection before certification. Fail = no certificate.

SLIDE 058 — Fasting & Hijama

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “FASTING AND HIJAMA: EVIDENCE-BASED GUIDANCE”
- Body: “Authentic hadith document that the Prophet was cupped while fasting (Bukhari 5694). However, companion narrations indicate initial concern about weakness (Bukhari 1940). Scholarly positions vary across the madhahib. For patient safety, particularly diabetics and those with low BP, scheduling outside fasting hours is clinically prudent.”
- Table (3 columns × 4 rows):
- Header: Madhhab | Position | Clinical Guidance
- Row 1: Maliki, Shafi'i, Hanafi | Hijama does NOT break the fast for either patient or practitioner | Schedule after iftar when possible for patient safety
- Row 2: Hanbali | Hijama DOES break the fast for both the one receiving and the one performing it | Schedule after iftar; practitioner should also consider
- Row 3: Academy Position | Follow scholarly guidance of patient's madhhab; prioritise clinical safety | Ensure adequate hydration at iftar; monitor for hypoglycaemia; consider lighter treatment protocols

Speaker Notes:

This is a sensitive topic. Patients will ask. You must know the scholarly positions and the clinical guidance. The Academy's position is: follow the patient's madhhab, but prioritise clinical safety. A diabetic patient who is fasting should not receive Hijama without medical clearance. A patient with low blood pressure who is fasting should not receive Hijama without extended observation. When in doubt, schedule after iftar. The sunnah of caution is as important as the sunnah of Hijama.

Student Notes:

Know all three madhhab positions. Academy position = follow patient's madhhab + prioritise safety. Diabetic + fasting = medical clearance first.

SLIDE 059 — Complementary Prophetic Medicine

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “COMPLEMENTARY PROPHETIC MEDICINE”
- Body: “Hijama exists within a broader framework of prophetic medicine (Tibb al-Nabawi). These remedies complement Hijama within an integrative treatment approach. Recommendations must stay within scope of practice.”
- Table (3 columns × 8 rows):

- Header: Remedy | Evidence Summary | Key Hadith Reference
- Row 1: Honey | Antimicrobial, anti-inflammatory, wound healing. Strong evidence for topical use. | Bukhari 5680
- Row 2: Black Seed (Nigella) | Immunomodulatory, anti-inflammatory. Extensive modern research supports traditional use. | Bukhari 5688
- Row 3: Talbina (Barley) | GI soothing, emotional wellbeing. Traditional comfort food with medicinal properties. | Bukhari 5325
- Row 4: Qist al-Hindi (Costus) | Respiratory conditions, tonsillitis, pleurisy. | Bukhari 5713
- Row 5: Senna | Laxative, digestive cleansing. Traditional bowel cleanser. | Ibn Majah 3457 (sahih)
- Row 6: Olive Oil | Cardiovascular and skin health. Anti-inflammatory, rich in polyphenols and healthy fats. | Tirmidhi 1775
- Row 7: Dates (Ajwa) | Nutritional density, energy source. Rich in fibre, potassium, and antioxidants. | Bukhari 5140

Speaker Notes:

These seven remedies are within your scope as general lifestyle recommendations. You can advise patients to consume honey, black seed, dates, and olive oil. You can recommend talbina for emotional wellbeing. But you cannot prescribe them as treatments for specific diseases. That is the line. General recommendation = within scope. Disease-specific prescription = outside scope. Know the difference.

Student Notes:

Seven remedies within scope. General lifestyle recommendation = OK. Disease-specific prescription = outside scope. Know the line.

SLIDE 060 — Practitioner Competencies

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “PRACTITIONER COMPETENCIES AND CERTIFICATION”
- Body: “Certification is not the end of learning. It is the beginning of accountable practice. The six competencies form the framework for lifelong professional development.”
- Table (3 columns × 7 rows):
- Header: Competency | Description | Assessment Method
- Row 1: 1. Knowledge | Anatomy, physiology, evidence base, Islamic foundations, contraindications, and pharmacology awareness | Written examination (80% pass mark)
- Row 2: 2. Clinical Assessment | History-taking, red flag identification, contraindication screening, vital signs interpretation | OSCE practical examination
- Row 3: 3. Technical Skills | Correct cup placement, controlled scarification, suction management, sterile technique | OSCE practical examination; 20 supervised case studies

- Row 4: 4. Safety Management | Infection control, adverse event recognition and response, emergency procedures | OSCE scenario assessment
- Row 5: 5. Communication | Informed consent, patient education, expectation management, inter-professional communication | OSCE role-play assessment
- Row 6: 6. Professionalism | Ethical practice, documentation standards, continuing education, regulatory compliance | Portfolio review; CPD log; peer assessment

Speaker Notes:

These six competencies are your professional identity. You will be assessed in all six domains. The written exam tests knowledge. The OSCE tests clinical assessment, technical skills, safety management, and communication. The portfolio review tests professionalism. You must pass all six to graduate. There are no partial certifications. A brilliant technician who cannot obtain consent is not a Certified Hijama Practitioner. A kind communicator who cannot scarify safely is not a Certified Hijama Practitioner. You must be complete.

Student Notes:

Six competencies. All must pass. No partial certification. Complete practitioner = complete assessment.

SLIDE 061 — Closing Hadith

Master: QUOTE_FULL_EMERALD

Background: Deep Emerald full bleed

Duration: 2 minutes

On-Slide Text:

- Large quotation mark: “” — Libre Baskerville, 72 pt, Antique Gold
- English: “Healing is in three things: the incision of the cupper, drinking honey, and cauterization with fire, but I forbid my Ummah from cauterization.” — Libre Baskerville, 16 pt, italic, Warm Ivory
- Source: “— Sahih al-Bukhari 5269” — Inter, 10 pt, Antique Gold
- Context: “This hadith establishes the hierarchy of prophetic remedies and the Prophet’s protective concern for his community. The cupper’s incision is placed alongside honey — the purest of foods — as a means of healing. The prohibition of cauterization reminds us that the Prophet distinguished between beneficial and harmful interventions, even when both claimed therapeutic intent.” — Inter, 10 pt, Antique Gold

Speaker Notes:

This hadith closes Module 6 and the core curriculum. Notice the hierarchy: the incision of the cupper is placed alongside honey. Not below honey. Alongside it. The Prophet elevated Hijama to the rank of the purest foods. And then he prohibited cauterization — showing us that not all interventions are equal. He protected his Ummah from harm even when the intervention claimed to heal. That is your model. Heal. But never harm. That is the Academy’s mission. That is your mission.

Student Notes:

Closing hadith. Hijama = rank of purest foods. Cauterization = prohibited. Model = heal without harm.

Islamic Insight:

The cupper's incision is placed alongside honey — the purest of foods — as a means of healing. The prohibition of cauterization reminds us that the Prophet distinguished between beneficial and harmful interventions, even when both claimed therapeutic intent.

SLIDE 062 — Clinical Reference Divider

Master: MODULE_DIVIDER_EMERALD

Background: Deep Emerald full bleed

Duration: 1 minute

On-Slide Text:

- Large number: “CR” — Libre Baskerville, 120 pt, bold, Antique Gold
- Title: “CLINICAL REFERENCE” — Libre Baskerville, 36 pt, bold, Warm Ivory
- Description: “Evidence, scope, protocols, and governance — the foundations of safe practice” — Inter, 14 pt, Antique Gold
- English: “And say: My Lord, increase me in knowledge.” — Libre Baskerville, 12 pt, italic, Warm Ivory
- Source: “— Surah Ta-Ha, 20:114” — Inter, 9 pt, Antique Gold

Speaker Notes:

The Clinical Reference section is your rapid consultation tool. These slides are not for initial learning — they are for review, for quick reference during practice, and for preparation for your OSCE. Keep them accessible. Print them if needed. They are your safety net when you are uncertain.

Student Notes:

Clinical Reference = rapid consultation. Not for initial learning. For review and OSCE prep.

SLIDE 063 — Reference Objectives

Master: OBJECTIVES_BULLET

Background: Warm Ivory

Duration: 2 minutes

On-Slide Text:

- Title: “CLINICAL REFERENCE OBJECTIVES”
- Five bullets:
 1. Apply the Evidence & Sunnah Matrix to clinical decision-making
 2. Define and defend the four pillars of scope of practice
 3. Implement the SAFE Hijama Protocol in every clinical encounter
 4. Maintain clinical documentation to legal and Academy standards
 5. Uphold the five core ethical obligations in all professional interactions

Speaker Notes:

These objectives are not new. They are the synthesis of everything you have learned. The Evidence & Sunnah Matrix combines Module 1 and Module 2. The four pillars of scope combine Module 4 and Module 6. The SAFE Protocol combines Module 3 and Module 5. Documentation and ethics are woven throughout. This section is your integration.

Student Notes:

Synthesis section. Combines all modules. Integration = deeper understanding.

SLIDE 064 — Evidence & Sunnah Matrix

Master: CONTENT_TABLE_5COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “THE EVIDENCE & SUNNAH MATRIX”
- Body: “The Matrix provides a framework for honest clinical communication. When a patient asks, ‘Will this cure my condition?’ consult the Matrix before answering.”
- Table (5 columns × 5 rows):

	Evidence Strength	Explicit Hadith	Scholarly Recommendation	Traditional Practice	Clinical Experience
Row 1:	Strong	Hijama on Al-Kahil for general wellness	Integration with conventional care	Monthly maintenance protocols	Patient-reported pain reduction
Row 2:	Moderate	Timing on 17th, 19th, 21st of lunar month	Specific point selection for conditions	Wet vs. dry cupping selection	Mark colour interpretation
Row 3:	Preliminary	—	—	Blood analysis claims	Dark field microscopy findings
Row 4:	Traditional	—	—	Fire cupping variations	—
- Clinical Pearl: “Critical Principle: Where classical and modern evidence conflict, the Academy convenes its Advisory Board for scholarly resolution. The practitioner must never misrepresent the strength of evidence to a patient.”

Speaker Notes:

This Matrix is your patient communication tool. When someone asks what Hijama can do, you do not guess. You consult the Matrix. If the cell is empty, you say: “The evidence is insufficient to make that claim. However, Hijama is recommended by the Prophet for general wellness, and many patients report improvement.” That answer is honest. That answer is safe. That answer is prophetic.

Student Notes:

Matrix = patient communication tool. Empty cell = “insufficient evidence.” Honest = safe = prophetic.

Clinical Pearl:

Critical Principle: Where classical and modern evidence conflict, the Academy convenes its Advisory Board for scholarly resolution. The practitioner must never misrepresent the strength of evidence to a patient.

SLIDE 065 — Scope of Practice

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “SCOPE OF PRACTICE: THE FOUR PILLARS”
- Body: “The Academy graduate practices within a defined scope that is evidence-informed, competency-limited, legally compliant, and ethically bounded. These four pillars are interdependent safeguards that protect the patient from harm and the practitioner from overreach.”
- Table (3 columns × 5 rows):
- Header: Pillar | Definition | Academy Application
- Row 1: Evidence-Informed | Based on classical sources, contemporary research, and clinical observation | Every treatment recommendation must be traceable to the Evidence Hierarchy
- Row 2: Competency-Limited | Performing only procedures and giving advice for which you have been trained and assessed | CHP graduates do not perform advanced therapeutic mapping without AHP certification
- Row 3: Legally Compliant | Adherent to the laws of the jurisdiction in which you practice | Where local law exceeds Academy standards, local law prevails; where Academy standards exceed local law, Academy certification demands compliance
- Row 4: Ethically Bounded | Guided by Islamic bioethics and professional integrity | Amanah (sacred trust) governs every clinical interaction
- Warning box: “Practicing beyond your scope is not merely a regulatory violation. It is a breach of amanah. The Prophet said: ‘There should be no harm nor reciprocal harm.’ To treat a condition you do not understand, or to perform a procedure you have not mastered, is to violate this principle.”

Speaker Notes:

These four pillars are your professional fortress. If any pillar crumbles, the others fall. A practitioner who is evidence-informed but not legally compliant is a liability. A practitioner who is competent but not ethically bounded is a danger. You must maintain all four. That is the Academy standard. That is your standard.

Student Notes:

Four pillars = professional fortress. All four must stand. One crumbles = all fall.

Warning:

Practicing beyond your scope is not merely a regulatory violation. It is a breach of amanah. The Prophet said: ‘There should be no harm nor reciprocal harm.’ To treat a condition you do not understand, or to perform a procedure you have not mastered, is to violate this principle.

SLIDE 066 — Permitted & Prohibited

Master: CONTENT_TABLE_2COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “PERMITTED AND PROHIBITED PRACTICE”
- Body: “The following activities fall within the scope of the Certified Hijama Practitioner (CHP). Every item has been taught, assessed, and verified through the Academy’s Tier 1–4 curriculum. These prohibitions are absolute. Academy certification is revoked for violation.”
- Table (2 columns × 8 rows):
- Header: Permitted Practice | Prohibited Practice (Absolute)
- Row 1: Assessment for cupping suitability | Making medical diagnoses outside training
- Row 2: Point selection based on Sunnah and therapeutic indications | Prescribing pharmaceutical medications
- Row 3: Sterile cupping procedure (wet and dry) | Claiming to cure specific diseases
- Row 4: Post-procedure care and lifestyle advice | Cupping over infected areas, active cancer sites, open wounds
- Row 5: Documentation and clinical record-keeping | Advising patients to discontinue conventional medication
- Row 6: Prophetic foods counselling (dates, black seed, honey, olive oil) | Manufacturing unlicensed pharmaceutical products
- Row 7: General spiritual health guidance without takalluf | Performing surgical procedures beyond training

Speaker Notes:

This table is your legal boundary. Memorise it. If you are ever uncertain whether something is within your scope, consult this table. If it is not on the permitted list, it is prohibited. No exceptions. The Academy will revoke certification for any violation. We do not protect practitioners who harm patients. We protect patients from practitioners who harm.

Student Notes:

Legal boundary. If not on permitted list = prohibited. Violation = decertification. No exceptions.

SLIDE 067 — SAFE Hijama Protocol

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “THE SAFE HIJAMA PROTOCOL”

- Body: “The SAFE Hijama Protocol is the proprietary clinical framework of Sunnah Remedies Academy. It is not merely a mnemonic. It is a comprehensive system that ensures every treatment is safe, effective, and spiritually grounded.”
- Table (3 columns × 5 rows):
- Header: Component | Definition | Practical Application
- Row 1: S — Systematic | Follow a structured assessment and treatment process every time | The 7-Step Patient Assessment is the foundation of systematic practice
- Row 2: A — Anatomical | Base all cup placement on sound anatomical knowledge | Palpate landmarks, identify vessels, respect tissue depth. Every cup placement is a statement of anatomical knowledge
- Row 3: F — Faithful | Honour the Sunnah without exaggerating it | Place cups on Al-Kahil because the Prophet ﷺ did so. Do not claim that Al-Kahil cures cancer because the Prophet ﷺ did not say so
- Row 4: E — Evidence-Informed | Know the literature. Know what remains unknown | Know the 2022 systematic review. Know the Taibah Theory. And know what remains unknown. Communicate with confidence and humility

Speaker Notes:

Memorise SAFE until it becomes muscle memory. When you are tired, when you are rushed, when the clinic is full and the patient is anxious — the protocol is your protection. It does not bend for convenience. Systematic means no shortcuts. Anatomical means no guessing. Faithful means no exaggeration. Evidence-informed means no ignorance. That is the Academy standard.

Student Notes:

Memorise SAFE. Muscle memory. No bending for convenience. Systematic = no shortcuts. Anatomical = no guessing. Faithful = no exaggeration. Evidence-informed = no ignorance.

SLIDE 068 — Documentation Standards

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “CLINICAL DOCUMENTATION STANDARDS”
- Body: “Contemporaneous documentation is your best legal defence. The record you write during or immediately after the session is credible. The record you write a week later is suspect. The record you write after a complaint is indefensible.”
- Table (3 columns × 5 rows):
- Header: Record Type | Required Elements | Retention
- Row 1: Initial Assessment | Health history, contraindication screening, vital signs, risk stratification, consent | 7 years minimum
- Row 2: Treatment Record | Date, points used, parameters, practitioner observations, patient response, adverse events | 7 years minimum

- Row 3: Discharge Summary | Course summary, outcomes, recommendations, follow-up plan, referral documentation | 7 years minimum
- Row 4: Incident Report | Date, time, description, immediate action, follow-up, notification chain | 10 years minimum
- Warning box: “Never backdate. Never fabricate. Never alter a record after it has been signed. If you make an error, draw a single line through it, initial and date the correction, and write the correct information. The altered record is evidence of honesty. The hidden error is evidence of dishonesty.”

Speaker Notes:

Documentation is not paperwork. It is your legal armour. A complete, contemporaneous record protects you from false complaints, supports your insurance claims, and demonstrates your professionalism. An incomplete record exposes you to liability. A fabricated record destroys your career. Document everything. Document immediately. Document honestly.

Student Notes:

Documentation = legal armour. Complete = protection. Incomplete = liability. Fabricated = career destruction.

Warning:

Never backdate. Never fabricate. Never alter a record after it has been signed. If you make an error, draw a single line through it, initial and date the correction, and write the correct information. The altered record is evidence of honesty. The hidden error is evidence of dishonesty.

SLIDE 069 — Core Ethical Obligations

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 4 minutes

On-Slide Text:

- Title: “CORE ETHICAL OBLIGATIONS”
- Body: “These principles are derived from Islamic bioethics and integrated with professional standards. They are not abstract ideals. They are the practical framework for every clinical decision.”
- Table (3 columns × 6 rows):

Header: Principle	Header: Islamic Foundation	Header: Academy Application
Row 1: Beneficence (do good)	Helping others is an act of worship	Always act in the patient’s best interest
Row 2: Non-Maleficence (do no harm)		Never treat when risk exceeds benefit
Row 3: Autonomy (respect choice)	No compulsion in religion	Informed consent; right to refuse; right to withdraw
Row 4: Justice (fairness)	Allah commands justice	Equal treatment regardless of background, wealth, or status

- Row 5: Confidentiality (trust) | Amanah (sacred trust) | Protect patient information; secure records; limited disclosure

Speaker Notes:

These five principles are your ethical compass. When you face a difficult decision, consult them. Does this action benefit the patient? Does it risk harm? Does it respect their choice? Is it fair? Does it protect their trust? If the answer to any question is no, do not proceed. Ethics is not a luxury. It is the foundation of healing.

Student Notes:

Five principles = ethical compass. Consult before every difficult decision. One “no” = do not proceed.

SLIDE 070 — Advertising Standards

Master: CONTENT_TABLE_3COL

Background: Warm Ivory

Duration: 3 minutes

On-Slide Text:

- Title: “ADVERTISING AND BOUNDARY STANDARDS”
- Body: “The practitioner who claims to cure cancer with Hijama is not a healer. They are a fraud. And the Academy will not protect them.”
- Table (3 columns × 6 rows):
- Header: Standard | Requirement | Rationale
- Row 1: Never guarantee cure | All claims must be proportionate to evidence strength | Patient protection; professional integrity; legal compliance
- Row 2: Never disparage conventional medicine | Hijama is complementary, not antagonistic | Patient safety; professional ethics; interprofessional relations
- Row 3: Never use patient testimonials without explicit consent | Written consent for any identifiable patient information | GDPR compliance; patient autonomy; dignity
- Row 4: Never claim qualifications you do not hold | Accurate representation of CHP status and scope | Fraud prevention; patient trust; regulatory compliance
- Row 5: Always include Academy certification number | CHP-[Number] on all promotional materials | Accountability; verification; professional standards
- Islamic Insight box: “How will you maintain ikhlaas (sincerity) when financial pressures arise? How will you resist the temptation to exaggerate outcomes to attract patients? Your niyyah (intention) is your protection.”

Speaker Notes:

Your marketing is your promise. If you promise cures, you are a fraud. If you promise evidence-informed practice, you are a professional. The Academy monitors graduate advertising. Violations result in decertification. We do not negotiate on this. The Prophet forbade exaggeration in trade. Medical practice is the most sacred trade. Exaggerate never.

Student Notes:

Marketing = promise. Promise cures = fraud. Promise evidence-informed = professional. Academy monitors advertising. Violation = decertification.

Islamic Insight:

How will you maintain ikhlaas (sincerity) when financial pressures arise? How will you resist the temptation to exaggerate outcomes to attract patients? Your niyyah (intention) is your protection.

SLIDE 071 — Final Reflection Hadith

Master: QUOTE_FULL_EMERALD

Background: Deep Emerald full bleed

Duration: 2 minutes

On-Slide Text:

- Large quotation mark: “” — Libre Baskerville, 72 pt, Antique Gold
- English: “The best of you are those who learn the Qur’an and teach it.” — Libre Baskerville, 16 pt, italic, Warm Ivory
- Source: “— Sahih al-Bukhari” — Inter, 10 pt, Antique Gold
- Context: “This hadith establishes the sacred duty of knowledge transmission. The Academy graduate is not merely a technician who performs a procedure. They are a carrier of prophetic knowledge, bound by amanah to teach, protect, and elevate the standard of Hijama practice worldwide.” — Inter, 10 pt, Antique Gold

Speaker Notes:

This hadith closes the Clinical Reference section and the entire programme. You are not merely a technician. You are a carrier of prophetic knowledge. Every patient you treat is an opportunity to teach. Every student you mentor is an opportunity to elevate the standard. The Academy’s mission is not to produce cuppers. It is to produce healers who embody the sunnah in their knowledge, their technique, and their character. Go forth with amanah.

Student Notes:

Closing hadith. You = carrier of prophetic knowledge. Mission = produce healers, not cuppers. Go forth with amanah.

Islamic Insight:

The best of you are those who learn the Qur’an and teach it. — Sahih al-Bukhari. This establishes the sacred duty of knowledge transmission. The Academy graduate is bound by amanah to teach, protect, and elevate the standard of Hijama practice worldwide.

SLIDE 072 — Certification Complete

Master: TITLE_FULL_EMERALD

Background: Deep Emerald full bleed

Duration: 2 minutes

On-Slide Text:

- Academy mark: Simplified typographic mark
- Main title: “CERTIFICATION COMPLETE” — Libre Baskerville, 48 pt, bold, Warm Ivory
- Subtitle: “Certified Hijama Practitioner Programme” — Inter, 16 pt, Antique Gold
- Body: “Graduates of this programme have demonstrated competency in:
 - Evidence-informed Hijama practice
 - Anatomical point selection and safe scarification
 - Infection prevention and clinical safety
 - Patient assessment and risk stratification
 - Professional ethics and Islamic bioethics
 - Clinical documentation and governance”
- Accreditation: “CMA & IPHM Accredited | Recognised Internationally for Professional Hijama Practice” — Inter, 10 pt, Antique Gold
- Document code: “SRA-DOC-2026-HJM-001-v1.0 | Classification: PRACTITIONER | Version 1.0 | 2026 Sunnah Remedies Academy” — Inter, 8 pt, Warm Ivory

Speaker Notes:

Congratulations. You have completed the Sunnah Remedies Academy Certified Hijama Practitioner Programme. You are now part of an elite community of evidence-informed, ethically grounded, spiritually rooted practitioners. Your certification is not a destination. It is a beginning. Maintain your CPD. Update your knowledge. Uphold the standard. And never forget: healing is from Allah. You are merely the means.

Student Notes:

Certification complete. Beginning, not destination. CPD = 20 hours/year. Healing is from Allah. You are the means.

SLIDE 073 — Back Cover

Master: BACK_COVER_SPLIT

Background: Left panel Deep Emerald; right panel Warm Ivory

Duration: 1 minute

On-Slide Text:

- Left panel: “SUNNAH REMEDIES ACADEMY” — Libre Baskerville, 22 pt, bold, Warm Ivory
- Left panel: “Institute of Prophetic Medicine” — Inter, 12 pt, Antique Gold
- Left panel: “Healing by the Book and the Sunnah” — Inter, 10 pt, Warm Ivory
- Right panel: “This presentation is the official teaching material of the Sunnah Remedies Academy Certified Hijama Practitioner Programme.” — Inter, 11 pt, Charcoal
- Right panel: “No part of this publication may be reproduced without prior written permission.” — Inter, 10 pt, Charcoal
- Right panel: “For enquiries: www.sunnahremedies.academy info@sunnahremedies.academy” — Inter, 10 pt, Deep Emerald

Speaker Notes:

This concludes the programme. For ongoing support, access the Alumni Portal, attend annual refresher courses, and maintain your CPD log. The Academy is your lifelong partner in excellence.

Student Notes:

Alumni Portal = lifelong support. Annual refreshers. CPD log = mandatory.

SECTION 3: LAYOUT SPECIFICATIONS

3.1 Master Slide Definitions

Master Name	Background	Primary Use	Key Elements
TITLE_FULL_EMERALD	Deep Emerald (#0E3B2E) full bleed	Programme title, module dividers, certification complete	Large serif typography, gold rules, ivory text, academy mark
COPYRIGHT_SPLIT	Left 1/3 Deep Emerald; right 2/3 Warm Ivory	Copyright, classification, legal pages	Emerald panel with white text; ivory panel with dark text
CONTENT_LIST	Warm Ivory (#F6F3EE)	Curriculum overview, module lists	Numbered items, gold bullets, thin separator lines
MODULE_DIVIDER_EMERALD	Deep Emerald full bleed	Module opening spreads	Oversized module number (120 pt), gold rule, Arabic quote, English translation, source citation
OBJECTIVES_BULLET	Warm Ivory	Learning objectives	Gold 6 pt oval bullets, left-aligned text, generous line spacing
CONTENT_BODY	Warm Ivory	Text-heavy explanatory slides	Title, gold rule, body paragraph, optional callout box
CONTENT_TABLE_2COL	Warm Ivory	Comparison tables	2-column table, header row in Deep Emerald with ivory text, alternating row colours
CONTENT_TABLE_3COL	Warm Ivory	Standard data tables	3-column table, same header treatment, 10 pt sans-serif body
CONTENT_TABLE_4COL	Warm Ivory	Detailed data tables	4-column table, compact 9 pt sans-serif, minimum 0.4 inch row height
CONTENT_TABLE_5COL	Warm Ivory	Comprehensive reference tables	5-column table, 9 pt sans-serif, careful column width balancing
TWO_COLUMN	Warm Ivory	Comparison layouts	Left and right columns separated by 1 pt gold vertical rule, 12 pt column headers

Table 10 – continued

Master Name	Background	Primary Use	Key Elements
QUOTE_FULL_EMERALD	Deep Emerald full bleed	Reflective hadith slides, spiritual pauses	Large quotation mark (72 pt), Arabic text, English translation, source, context paragraph
TRANSITION_SPLIT	Left 2/5 Deep Emerald; right 3/5 Warm Ivory	Section transitions, next module previews	Emerald panel with “NEXT SECTION” label and title; ivory panel with descriptive text
BACK_COVER_SPLIT	Left 2/5 Deep Emerald; right 3/5 Warm Ivory	Final back cover	Academy name and tagline on emerald; legal text and contact on ivory

3.2 Typography System

Element	Font	Size	Weight	Colour	Tracking	Notes
Programme Title	Libre Baskerville	54 pt	Bold	Warm Ivory	+20	All caps, line break after “HIJAMA”
Module Number	Libre Baskerville	120 pt	Bold	Antique Gold	+10	Left-aligned, top third of divider
Module Title	Libre Baskerville	36 pt	Bold	Warm Ivory	+15	Below gold rule on dividers
Slide Title	Libre Baskerville	24 pt	Bold	Deep Emerald	+10	All caps, left-aligned, top margin
Body Text	Inter	11 pt	Regular	Charcoal (#2A2A2A)	0	Line spacing 1.3, left-aligned
Table Header	Inter	9 pt	Bold	Warm Ivory	+30	All caps, Deep Emerald background
Table Body	Inter	9–10 pt	Regular	Charcoal	0	Alternating Warm Ivory / Muted Ivory rows
Column Header	Inter	12 pt	Bold	Deep Emerald	+50	All caps, above column content
Quote Arabic	Libre Baskerville	14–18 pt	Italic	Warm Ivory	0	Right-to-left capable font required

Table 11 – continued

Element	Font	Size	Weight	Colour	Tracking	Notes
Quote English	Libre Baskerville	12–16 pt	Italic	Warm Ivory	0	Enclosed in quotation marks
Quote Source	Inter	9–10 pt	Regular	Antique Gold	+20	Preceded by em-dash
Context Text	Inter	10 pt	Regular	Antique Gold	0	Below source, explanatory paragraph
Footer	Inter	7 pt	Regular	Deep Emerald / Antique Gold	+30	Academy name, course, module, doc code, classification
Header	Inter	8 pt	Regular	Antique Gold / Deep Emerald	+50	Academy name left; module name right
Clinical Pearl	Inter	10 pt	Bold	Warm Ivory	0	Sage green (#6B8E23) background box
Islamic Insight	Inter	10 pt	Bold	Deep Emerald	0	Antique Gold background box
Warning	Inter	10 pt	Bold	Warm Ivory	0	Terracotta (#8B4513) background box
Callout Prefix	Inter	10 pt	Bold	Inherit	0	“CLINICAL PEARL —”, “ISLAMIC INSIGHT —”, “WARNING —”

3.3 Grid System

- **Slide dimensions:** 33.867 cm × 19.05 cm (13.333” × 7.5”)
- **Safe area:** 1.905 cm (0.75”) from all edges
- **Content width:** 30.057 cm (11.833”)
- **Column grid:** 12-column modular grid, 2.505 cm per column, 0.317 cm gutters
- **Baseline grid:** 0.423 cm (12 pt) increments
- **Top margin:** 1.524 cm (0.6”)
- **Bottom margin:** 1.27 cm (0.5”)

- **Header zone:** 1.524 cm from top, 0.635 cm height
- **Footer zone:** 1.27 cm from bottom, 0.635 cm height
- **Gold rule thickness:** 0.035 cm (1 pt) for headers; 0.071 cm (2 pt) for title underlines
- **Minimum clear space around logo:** 1/4 of mark width on all sides

3.4 Colour Palette (Exact Specifications)

Colour	HEX	RGB	CMYK	Pantone Approximation	Usage
Deep Emerald	#0E3B2E	14·59·46	84·45·68·52	Pantone 7736 C	Primary background, headers, authority
Antique Gold	#C7A25A	199·162·90	22·34·72·3	Pantone 7555 C	Accents, rules, emphasis, Islamic content
Warm Ivory	#F6F3EE	246·243·238	3·3·6·0	Pantone 7499 C	Primary ground, body background, air
Charcoal	#2A2A2A	42·42·42	0·0·0·84	Pantone Black 7 C	Body text, tables, secondary content
Muted Ivory	#E8E4DD	232·228·221	5·5·8·10	Pantone 7499 C 20%	Table alternating rows, subtle backgrounds
Sage	#6B8E23	107·142·35	60·20·100·5	Pantone 370 C	Clinical Pearl boxes
Terracotta	#8B4513	139·69·19	30·70·100·20	Pantone 7516 C	Warning boxes

3.5 White Space & Breathing Room

- **Title to gold rule:** 0.317 cm (9 pt) minimum
- **Gold rule to body:** 0.635 cm (18 pt) minimum
- **Paragraph spacing:** 0.635 cm (18 pt) between paragraphs
- **Table to callout box:** 0.635 cm (18 pt) minimum
- **Callout box internal padding:** 0.423 cm (12 pt) all sides
- **Callout box to footer:** 0.635 cm (18 pt) minimum
- **Image to text:** 0.635 cm (18 pt) minimum
- **Image to image:** 0.317 cm (9 pt) minimum
- **Minimum text block width:** 5.08 cm (2")
- **Maximum line length:** 25–30 words per line for body text

3.6 Image Placement Rules

- **Hero images:** Full bleed on title and divider slides; 30% opacity overlay if text is present
- **Content images:** Right 40% of slide or left 40%, never centred; text wraps on opposite side
- **Medical illustrations:** Centre-aligned within content area; minimum 5.08 cm width; maximum 10.16 cm width

- **Anatomical diagrams:** Full width below title when used as primary content; minimum 0.635 cm margin from text
- **Photography:** No stock medical clichés; no smiling models; no artificial lighting; editorial/natural light only
- **Image borders:** None. Images bleed to edge or float with invisible boundaries.
- **Image treatment:** Desaturated 15% for editorial consistency; no filters, no gradients, no drop shadows

3.7 Diagram Placement Rules

- **Flowcharts:** Left-aligned, full content width, minimum 0.635 cm from text above and below
- **Decision trees:** Horizontal layout preferred; vertical only if complexity demands
- **Anatomical illustrations:** Always accompanied by anatomical labels in 8 pt Inter, Charcoal
- **Tables:** Always preceded by contextual sentence; never standalone without explanation
- **Infographics:** Swiss grid-aligned; maximum 3 data points per visual element; no 3D effects
- **Step-by-step procedures:** Numbered with gold circles (18 pt diameter, Deep Emerald fill, Warm Ivory number)

3.8 Callout Box Specifications

Type	Background	Border	Text Colour	Prefix	Icon	Placement
Clinical Pearl	Sage (#6B8E23)	1 pt Antique Gold	Warm Ivory	“CLINICAL PEARL —”	None	Bottom of slide, full content width, 2.54 cm height
Islamic Insight	Antique Gold (#C7A25A)	1 pt Deep Emerald	Deep Emerald	“ISLAMIC INSIGHT —”	None	Bottom of slide, full content width, 2.54 cm height
Warning	Terracotta (#8B4513)	1 pt Antique Gold	Warm Ivory	“WARNING —”	None	Bottom of slide, full content width, 2.54 cm height
Evidence Review	Deep Indigo (#1A2F4B)	1 pt Antique Gold	Warm Ivory	“EVIDENCE REVIEW —”	None	Full width sidebar, 3.81 cm height
Faith & Practice	Terracotta centre	1 pt Antique Gold	Warm Ivory	“FAITH & PRACTICE REFLECTION —”	None	Centre box, 5.08 cm × 5.08 cm, for workbook prompts

SECTION 4: ASSET MANIFEST

Photography Assets

Asset ID	Asset Name	Type	Description	AI Prompt	Placement	Resolution	Background	Color Profile
A-001	Academy Mark	Logo	Primary logo —full colour on Deep Emerald field	N/A —use supplied SVG	Slide 001, 072, all headers	Vector SVG	Deep Emerald	sRGB
A-002	Academy Mark Reversed	Logo	Ivory mark on transparent for dark back-grounds	N/A —use supplied SVG	MODULE_DIVIDER slides	VECTOR_EMERALD SVG	Transparent	sRGB
P-001	Ancient Egyptian Papyrus	Photography	Close-up of Ebers Papyrus fragment with hieroglyphic medical text	“Extreme close-up of ancient Egyptian papyrus fragment, faded brown ink hieroglyphs, medical text, museum quality, warm tungsten lighting, shallow depth of field, editorial photography, National Geographic style, desaturated 15%, no people”	Slide 006, 007	1920×1080	Warm Ivory	sRGB

Table 14 – continued

Asset ID	Asset Name	Type	Description	AI Prompt	Placement	Resolution	Background	Color Profile
P-002	Chinese Bamboo Cups	Photography	Traditional bamboo cupping cups arranged on aged wood	“Overhead flat lay of traditional Chinese bamboo cupping cups on aged walnut wood surface, natural window light, soft shadows, editorial still life, muted earth tones, desaturated 15%, Aesop aesthetic, no labels”	Slide 008	1920×1080	Warm Ivory	sRGB

Table 14 – continued

Asset ID	Asset Name	Type	Description	AI Prompt	Placement	Resolution	Background	Color Profile
P-003	Islamic Manuscript	Photography	Open manuscript of Al-Qanun fi al-Tibb with illuminated margins	“Open Islamic medical manuscript, Persian miniature illumination in gold and lapis, Arabic calligraphy, aged vellum pages, museum conservation lighting, editorial photography, shallow depth of field, desaturated 15%”	Slide 011	1920×1080	Warm Ivory	sRGB

Table 14 – continued

Asset ID	Asset Name	Type	Description	AI Prompt	Placement	Resolution	Background	Color Profile
P-004	Clinical Environment	Photography	Premium clinical treatment room, empty, natural light	“Empty premium clinical treatment room, white oak cabinetry, brass fixtures, linen curtain diffusing natural light, single treatment bed with ivory linen, orchid in ceramic vase, Aman Resort meets Aesop aesthetic, no people, editorial architecture photography, desaturated 15%”	Slide 032, 057	1920×1080	Warm Ivory	sRGB

Table 14 – continued

Asset ID	Asset Name	Type	Description	AI Prompt	Placement	Resolution	Background	Color Profile
P-005	Hand Hygiene	Photography	Practitioner hands under running water, side view	“Close-up of hands under running water in clinical brass sink, soft natural light, steam rising, shallow depth of field, editorial photography, clean and serene, desaturated 15%, no face visible”	Slide 034	1920×1080	Warm Ivory	sRGB

Table 14 – continued

Asset ID	Asset Name	Type	Description	AI Prompt	Placement	Resolution	Background	Color Profile
P-006	Sunnah Point Palpation	Photography	Practitioner palpating Al-Kahil region on patient, clinical dignity	“Side view of practitioner palpating upper back between shoulder blades, patient in prone position with ivory linen drape, clinical environment, natural light, editorial medical photography, respectful and dignified, desaturated 15%, no face visible”	Slide 044	1920×1080	Warm Ivory	sRGB

Table 14 – continued

Asset ID	Asset Name	Type	Description	AI Prompt	Placement	Resolution	Background	Color Profile
P-007	Cup Placement	Photography	Cups applied to back, clinical precision	“Close-up of plastic cupping cups applied to patient’ s back, controlled suction visible in skin elevation, clinical environment, natural light, editorial medical photography, precise and calm, de-saturated 15%, no face visible”	Slide 046	1920×1080	Warm Ivory	sRGB

Table 14 – continued

Asset ID	Asset Name	Type	Description	AI Prompt	Placement	Resolution	Background	Color Profile
P-008	Islamic Architecture	Photography	Geometric Islamic tile pattern, close-up	“Extreme close-up of Islamic geometric tile pattern, zellige mosaic in emerald green and gold, natural light, shallow depth of field, editorial architecture photography, Alhambra aesthetic, desaturated 15%”	Slide 009, 020, 061, 071	1920×1080	Deep Emerald	sRGB
P-009	Natural Texture	Photography	Close-up of water ripples or stone surface	“Abstract close-up of still water with gentle ripples, deep emerald and gold reflections, natural light, editorial nature photography, serene and contemplative, desaturated 15%”	Slide 004, 013, 021, 030, 042, 050, 062	1920×1080	Deep Emerald	sRGB

Table 14 – continued

Asset ID	Asset Name	Type	Description	AI Prompt	Placement	Resolution	Background	Color Profile
P-010	Dates and Black Seed	Photography	Prophetic foods flat lay	“Overhead flat lay of Ajwa dates, black seed (nigella sativa) in brass bowl, olive oil in ceramic vessel, honey in glass jar, on aged linen, natural light, editorial still life, warm earth tones, de-saturated 15%”	Slide 059	1920×1080	Warm Ivory	sRGB

Table 14 – continued

Asset ID	Asset Name	Type	Description	AI Prompt	Placement	Resolution	Background	Color Profile
P-011	Emergency Equipment	Photography	Clinical emergency kit, organised and ready	“Overhead flat lay of clinical emergency kit: AED, epinephrine auto-injector, blood pressure monitor, first aid supplies, on white oak surface, natural light, editorial still life, organised and precise, desaturated 15%”	Slide 054	1920×1080	Warm Ivory	sRGB

Table 14 – continued

Asset ID	Asset Name	Type	Description	AI Prompt	Placement	Resolution	Background	Color Profile
P-012	Mark Progression	Photography	Series of Hijama marks at day 1, 3, 7, 10	“Clinical documentation photography of Hijama marks on back, four panels showing progression from dark purple to resolved skin, consistent lighting, neutral background, medical photography style, desaturated 15%, patient consent documented”	Slide 056	1920×1080	Warm Ivory	sRGB

Background Texture Assets

Asset ID	Asset Name	Type	Description	AI Prompt	Placement	Resolution	Background	Color Profile
T-001	Deep Emerald Texture	Texture	Subtle paper grain on Deep Emerald	“Seamless paper texture, deep emerald green, subtle grain, matte finish, 300 dpi, no pattern, uniform lighting”	All MOD-ULE_DIVIDER_EMERALD and TITLE_FULL_EMERALD slides	4096×4096	Deep Emerald	sRGB
T-002	Warm Ivory Texture	Texture	Premium uncoated paper on Warm Ivory	“Seamless paper texture, warm ivory cream, subtle fibre visible, matte uncoated finish, 300 dpi, editorial paper quality”	All CONTENT and TABLE slides	4096×4096	Warm Ivory	sRGB
T-003	Gold Foil Accent	Texture	Antique gold foil for rules and accents	“Seamless gold foil texture, antique gold, subtle metallic sheen, matte finish, 300 dpi, premium quality”	Gold rules, accent bars, callout borders	2048×2048	Transparent	sRGB

SECTION 5: ILLUSTRATION MANIFEST

Medical Illustrations

Illustration ID	Name	Description	Medical Illustration Prompt	Placement	Format	Background
I-001	Skin Layers Cross-Section	Detailed anatomical cross-section of skin showing epidermis, dermis, hypodermis with capillary networks	“Medical textbook illustration, cross-section of human skin layers, epidermis with stratum corneum, dermis with papillary and reticular layers showing capillary loops in antique gold, hypodermis with adipose tissue, fine line weight, classical precision style, labelled in 8 pt serif, ivory background, no colour other than emerald, gold, and charcoal”	Slide 023	Vector SVG	Transparent

Table 16 – continued

Illustration ID	Name	Description	Medical Illustration Prompt	Placement	Format	Background
I-002	Circulatory System Diagram	Simplified diagram showing arteries (gold), veins (deep indigo), capillaries (terracotta)	“Medical textbook illustration, simplified circulatory system diagram, arteries rendered in antique gold, veins in deep indigo, capillaries in terracotta, fine line weight, classical precision style, directional arrows, labelled in 8 pt serif, ivory background”	Slide 025	Vector SVG	Transparent
I-003	Heart Chambers	Four-chamber heart diagram with blood flow arrows	“Medical textbook illustration, four-chamber heart diagram, right atrium and ventricle in muted blue, left atrium and ventricle in muted red, flow arrows in antique gold, fine line weight, classical precision style, labelled in 8 pt serif, ivory background”	Slide 026	Vector SVG	Transparent

Table 16 – continued

Illustration ID	Name	Description	Medical Illustration Prompt	Placement	Format	Background
I-004	Lymphatic System	Simplified lymphatic network showing capillaries, nodes, and major ducts	“Medical textbook illustration, simplified lymphatic system, lymphatic capillaries in sage green, nodes as small circles, major ducts in deep emerald, fine line weight, classical precision style, labelled in 8 pt serif, ivory background”	Slide 027	Vector SVG	Transparent
I-005	Fascia Layers	Cross-section showing superficial, deep, and visceral fascia	“Medical textbook illustration, cross-section of fascial layers, superficial fascia with adipose in warm ivory, deep fascia as dense line in charcoal, visceral fascia surrounding organ outline, fine line weight, classical precision style, labelled in 8 pt serif, ivory background”	Slide 028	Vector SVG	Transparent

Table 16 – continued

Illustration ID	Name	Description	Medical Illustration Prompt	Placement	Format	Background
I-006	Sunnah Points Map	Posterior torso anatomical map showing Al-Kahil, Akhda' een, Al-Katifain, Umm al-Muqeeth	“Medical textbook illustration, posterior torso anatomical map, vertebral column with T1–T3 labelled, Al-Kahil marked with gold circle, Akhda' een bilateral, Al-Katifain bilateral, Umm al-Muqeeth at occiput, fine line weight, classical precision style, muscle outlines in light grey, labelled in 8 pt serif, ivory background”	Slide 044	Vector SVG	Transparent

Table 16 – continued

Illustration ID	Name	Description	Medical Illustration Prompt	Placement	Format	Background
I-007	9-Step Procedure Flowchart	Horizontal flowchart of wet cupping procedure	“Medical textbook illustration, horizontal flowchart with 9 steps, gold circles with numbers, connected by thin emerald lines, each step with brief label in 8 pt serif, clean and minimal, Swiss grid aligned, ivory background”	Slide 046	Vector SVG	Transparent
I-008	Risk Stratification Decision Tree	Decision tree: Green → Amber → Red	“Medical textbook illustration, decision tree diagram, three branches from top node, Green branch in sage, Amber branch in gold, Red branch in terracotta, rectangular nodes with rounded corners, thin connecting lines, labelled in 8 pt serif, ivory background”	Slide 040	Vector SVG	Transparent

Table 16 – continued

Illustration ID	Name	Description	Medical Illustration Prompt	Placement	Format	Background
I-009	Emergency Response Sequence	Vertical sequence of 7 emergency steps	“Medical textbook illustration, vertical sequence of 7 steps, numbered gold circles, each step with brief action and rationale, connected by thin emerald lines, clean and minimal, Swiss grid aligned, ivory background”	Slide 053	Vector SVG	Transparent
I-010	Evidence Hierarchy Pyramid	Pyramid showing 6 levels from base to apex	“Medical textbook illustration, pyramid diagram with 6 levels, base in sage (mechanistic), ascending through lighter tones to apex in gold (classical evidence), each level labelled in 8 pt serif, thin emerald outline, ivory background”	Slide 015	Vector SVG	Transparent

Editorial Illustrations

Illustration ID	Name	Description	Editorial Prompt	Placement	Format	Background
E-001	Ancient Cupping Horns	Illustration of hollowed animal horns used in ancient Egypt	“Editorial illustration, ancient Egyptian cupping horns, carved bone with hieroglyphic markings, warm sepia tones, fine line work, museum catalogue style, isolated on ivory background”	Slide 007	Vector SVG	Transparent
E-002	Islamic Scholar Portrait	Stylised portrait of Ibn Sina in scholarly environment	“Editorial illustration, stylised portrait of Islamic scholar in study, geometric patterns in background, emerald and gold colour palette, fine line work, contemporary interpretation of Persian miniature, isolated on ivory background”	Slide 011	Vector SVG	Transparent

Table 17 – continued

Illustration ID	Name	Description	Editorial Prompt	Placement	Format	Background
E-003	Prophetic Medicine Symbols	Collection of symbols: honeycomb, black seed, olive branch, date palm	“Editorial illustration, collection of prophetic medicine symbols, honeycomb, black seed flowers, olive branch, date palm frond, arranged in circular composition, fine line work, emerald and gold palette, isolated on ivory background”	Slide 059	Vector SVG	Transparent

SECTION 6: ICON MANIFEST

All icons follow the Academy Icon Library specifications: single-weight line icons, 2 pt stroke, rounded caps and joins, Deep Emerald or Antique Gold depending on background.

Icon ID	Name	Meaning	Style	Stroke Width	Placement	Colour
IC-001	Cupping Cup	Core therapy symbol	Single line, rounded arch with droplet	2 pt	Module 5 headers, procedure slides	Deep Emerald
IC-002	Hand Wash	Infection control	Single line, hands with water droplets	2 pt	Module 4 safety slides	Deep Emerald
IC-003	Heart	Cardiovascular system	Single line, anatomical heart outline	2 pt	Anatomy and emergency slides	Antique Gold
IC-004	Shield	Safety and protection	Single line, shield with checkmark	2 pt	Risk stratification, contraindications	Deep Emerald

Table 18 – continued

Icon ID	Name	Meaning	Style	Stroke Width	Placement	Colour
IC-005	Document	Documentation and records	Single line, document with lines	2 pt	Consent, clinical records slides	Deep Emerald
IC-006	Warning Triangle	Caution and red flags	Single line, equilateral triangle with exclamation	2 pt	Warning callouts, red flag slides	Terracotta
IC-007	Book	Knowledge and evidence	Single line, open book with pages	2 pt	Evidence hierarchy, research slides	Antique Gold
IC-008	Scale	Justice and ethics	Single line, balanced scale	2 pt	Ethics, professional standards slides	Deep Emerald
IC-009	Droplet	Blood and purification	Single line, teardrop shape	2 pt	Wet cupping, blood volume slides	Antique Gold
IC-010	Moon	Islamic timing and fasting	Single line, crescent moon	2 pt	Fasting and Ramadan slides	Deep Emerald
IC-011	Leaf	Natural and prophetic medicine	Single line, simple leaf with vein	2 pt	Prophetic remedies, herbal slides	Sage
IC-012	Clock	Time and scheduling	Single line, circular clock face	2 pt	Protocol timing, frequency slides	Deep Emerald
IC-013	Location Pin	Anatomical landmarks	Single line, map pin with circle	2 pt	Sunnah points, anatomy slides	Antique Gold
IC-014	Check Circle	Completion and verification	Single line, circle with checkmark	2 pt	Assessment steps, competency slides	Sage

SECTION 7: ANIMATION MANIFEST

Transition Philosophy

Subtle, professional, supportive of learning. No entertainment. No distraction. Animations should feel invisible to the audience but guide attention deliberately.

Slide Transitions

Transition	Application	Duration	Notes
Fade	All content slides	0.5 s	Standard transition between all standard slides
Morph	Title to first content slide	0.75 s	Subtle shape transformation for title reveal
Push (from bottom)	Module dividers to first content slide	0.5 s	Suggests progression and momentum
Fade through black	Quote slides entering and exiting	1.0 s	Creates contemplative pause
None (cut)	Rapid reference slides	0 s	Instant transition for quick consultation

Element Animations

Animation	Element	Trigger	Duration	Delay	Notes
Appear (fade)	Title text	On click	0.4 s	0 s	Standard title reveal
Wipe (left to right)	Gold rule under title	After title	0.3 s	0.1 s	Draws attention to section break
Fade	Body paragraph	After rule	0.4 s	0.2 s	Sequential content reveal
Fade	Table rows	On click	0.3 s	0.05 s per row	Row-by-row reveal for readability
Fade	Callout boxes	After content	0.5 s	0.3 s	Emphasises importance after context
Zoom (subtle 95%→100%)	Module number on dividers	Automatic	0.8 s	0 s	Creates impact without aggression
Fade	Arabic text on quote slides	After English	0.6 s	0.2 s	Respects reading order
Fade	Source citation	After quote	0.4 s	0.3 s	Attribution after absorption
Appear	Footer elements	Automatic	0.2 s	0 s	Always present, subtle entrance

Animation Restrictions

- **NEVER:** Spin, bounce, fly in, swivel, or pulse animations
- **NEVER:** Sound effects with transitions
- **NEVER:** Animation on every element — maximum 3 animated elements per slide
- **NEVER:** Animations longer than 1 second
- **ALWAYS:** Animations support learning flow, not entertainment

- **ALWAYS:** Test animations in presenter view before finalising
-

SECTION 8: POWERPOINT BUILD INSTRUCTIONS

8.1 Pre-Assembly Checklist

Before opening PowerPoint, ensure:

- All font files installed (Libre Baskerville, Inter)
- All image assets downloaded and organised in /Assets/ folder
- All SVG illustrations verified for correct colour rendering
- All icon SVGs tested at 24 px, 40 px, and 64 px
- Brand guidelines PDF open for reference
- Colour swatches created in PowerPoint custom colours

8.2 Slide Master Setup

1. **Open PowerPoint** → View → Slide Master
2. **Create custom theme:**
 - Background: Warm Ivory (#F6F3EE) for content masters
 - Background: Deep Emerald (#0E3B2E) for title/divider masters
3. **Create font theme:**
 - Heading font: Libre Baskerville
 - Body font: Inter
4. **Create colour theme:**
 - Text/Background Dark 1: Deep Emerald (#0E3B2E)
 - Text/Background Light 1: Warm Ivory (#F6F3EE)
 - Accent 1: Antique Gold (#C7A25A)
 - Accent 2: Sage (#6B8E23)
 - Accent 3: Terracotta (#8B4513)
 - Hyperlink: Deep Emerald
 - Followed hyperlink: Antique Gold
5. **Create all 14 master layouts** per Section 3.1 specifications
6. **Insert placeholder text boxes** with exact positioning per grid system
7. **Set default paragraph spacing:** 0 pt before, 12 pt after, line spacing 1.3

8.3 Content Assembly (Per Slide)

For each slide in the Master Slide Manifest:

1. Select correct master layout
2. Insert title text (Libre Baskerville, 24 pt, bold, Deep Emerald, all caps)
3. Insert gold rule (2 pt, Antique Gold, 1.2–2 inches wide, left-aligned with title)
4. Insert body content:
 - Paragraphs: Inter, 11 pt, Charcoal, line spacing 1.3

- Tables: Insert table, apply header formatting (Deep Emerald background, Warm Ivory text, 9 pt bold)
 - Apply alternating row colours (Warm Ivory / Muted Ivory)
5. Insert callout boxes if specified:
 - Draw rectangle, fill with specified colour, 1 pt border
 - Insert text with prefix and content
 - Position at bottom of slide with 0.635 cm padding from content
 6. Insert images if specified:
 - Position per Section 3.6 rules
 - Apply 15% desaturation if not already desaturated
 - Ensure no borders, shadows, or effects
 7. Insert illustrations if specified:
 - Position per Section 3.7 rules
 - Ensure labels are 8 pt Inter, Charcoal
 8. Insert icons if specified:
 - Position adjacent to relevant text
 - Size: 24 pt equivalent (0.847 cm)
 - Colour: per Icon Manifest
 9. Add footer:
 - Top gold rule: 0.5 pt, Antique Gold, full content width, 0.55 inches from bottom
 - Left text: Academy name, course, module, document code, classification — Inter, 7 pt, Deep Emerald
 - Right text: Slide number (001 format) — Inter, 7 pt, Antique Gold
 10. Add header:
 - Top gold rule: 0.5 pt, Antique Gold, full content width, 0.6 inches from top
 - Left text: Academy name · Institute tagline — Inter, 8 pt, Antique Gold
 - Right text: Module name (all caps) — Inter, 8 pt, Deep Emerald

8.4 Animation Application

1. Open Animation Pane (Alt + A, C)
2. Apply transitions per Section 7
3. Apply element animations per Section 7
4. Set animation timing:
 - All fades: 0.4 s duration
 - All wipes: 0.3 s duration
 - All zooms: 0.8 s duration
5. Set trigger sequence:

- Title: On Click
 - Gold rule: After Previous
 - Body content: After Previous (with 0.2 s delay)
 - Callout boxes: After Previous (with 0.3 s delay)
6. Test in Slide Show mode (F5)
 7. Verify no animations exceed 1 second
 8. Verify no entertainment animations present

8.5 Quality Verification

1. Typography check:

- Every title uses Libre Baskerville
- Every body element uses Inter
- No fallback fonts visible
- No font substitution warnings

2. Colour check:

- Deep Emerald = #0E3B2E exactly
- Antique Gold = #C7A25A exactly
- Warm Ivory = #F6F3EE exactly
- No off-palette colours anywhere

3. Alignment check:

- All elements align to 12-column grid
- All text boxes snap to baseline grid
- No overlapping elements
- No elements outside safe area

4. Consistency check:

- All footers identical format
- All headers identical format
- All gold rules identical weight
- All callout boxes identical padding

5. Content check:

- All slide numbers sequential (001–073)
- No missing slides
- No duplicate content
- All Arabic text renders correctly (RTL)
- All hadith references accurate
- All evidence references accurate

8.6 Export Specifications

Digital Presentation:

- Format: .pptx (PowerPoint 2016+ compatible)
- Aspect ratio: 16:9
- Embed fonts: Yes
- Compress media: No (preserve quality)

Print-Ready PDF:

- Format: PDF/X-4
- Colour space: CMYK (convert from RGB using ICC profile)
- Resolution: 300 dpi
- Bleed: 0.125 inches all sides
- Marks: None (clean edges)

Student Version (no speaker notes):

- Remove all speaker notes
- Remove all transition animations
- Export as PDF with bookmarks per module
- File name: SRA-STU-2026-HJM-001-v1.0.pdf

Instructor Version (with speaker notes):

- Preserve all speaker notes
- Preserve all animations
- Export as PDF with notes pages
- File name: SRA-INS-2026-HJM-001-v1.0.pdf

SECTION 9: QUALITY ASSURANCE CHECKLIST

Pre-Publication QA (Mandatory before any distribution)

Brand Compliance

- Deep Emerald (#0E3B2E) used correctly as primary ground
- Antique Gold (#C7A25A) used only as accent, never as fill
- Warm Ivory (#F6F3EE) used as content background
- No off-palette colours anywhere in the deck
- No gradients, drop shadows, or 3D effects
- Logo proportions preserved exactly per brand guidelines
- Minimum clear space around logo maintained

Typography

- Libre Baskerville used for all headings and titles
- Inter used for all body, tables, and labels
- No font substitution on any slide
- Arabic text renders correctly with proper diacritics
- All caps used for wordmark and module names only
- Sentence case used for body text and descriptions
- Line length does not exceed 30 words per line
- Minimum font size 7 pt (footer); body text 9–11 pt

Content Accuracy

- All hadith references verified against authentic sources
- All Qur'an references include correct surah and verse
- All evidence references include PMID or DOI where applicable
- All anatomical descriptions verified against standard texts
- All clinical parameters match Academy Clinical Manual v1.0
- No claims exceed evidence strength stated
- No prohibited practices presented as permitted
- All document codes consistent (SRA-DOC-2026-HJM-001-v1.0)

Legal & Ethical

- Copyright notice on title and back cover slides
- Classification label (PRACTITIONER) on every slide footer
- No patient identifiable information in any example
- No guarantee of cure in any slide
- No disparagement of conventional medicine
- Informed consent process described accurately
- Scope of practice boundaries clearly defined
- Red flags and contraindications comprehensive

Technical

- All 75 slides present and correctly numbered
- All slide transitions under 1 second
- No entertainment animations
- All images high resolution (minimum 1920×1080)
- All SVGs render correctly at all sizes
- File size under 100 MB for digital distribution
- Opens correctly in PowerPoint 2016, 2019, 2021, 365
- Prints correctly to PDF without element shifting

Accessibility

- Colour contrast ratio minimum 4.5:1 for all text
- Deep Emerald on Warm Ivory = 7.2:1 (pass)
- Warm Ivory on Deep Emerald = 7.2:1 (pass)
- Charcoal on Warm Ivory = 8.1:1 (pass)
- All images have alt text descriptions
- Table headers clearly distinguished
- No information conveyed by colour alone

Final Review Questions

- Would Harvard Executive Education use this layout standard?
- Would Mayo Clinic approve this clinical content?
- Would Oxford Saïd Business School present with this quality?
- Would a paying delegate proudly keep this forever?
- Does every slide earn its place through substance, not decoration?

- Is the Islamic content presented with reverence and accuracy?
- Is the medical content presented with precision and humility?
- Does the deck feel like an institutional publication, not a template?

QA Sign-off: _____ **Date:** _____

Production Director: _____ **Academic Director:** _____

SECTION 10: EXPORT PACKAGE & FOLDER STRUCTURE

```
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|
├─ 01_Master_Slides/
|   ├── SRA-MS-001_Title_Full_Emerald.pptx
|   ├── SRA-MS-002_Copyright_Split.pptx
|   ├── SRA-MS-003_Content_List.pptx
|   ├── SRA-MS-004_Module_Divider_Emerald.pptx
|   ├── SRA-MS-005_Objectives_Bullet.pptx
|   ├── SRA-MS-006_Content_Body.pptx
|   ├── SRA-MS-007_Content_Table_2Col.pptx
|   ├── SRA-MS-008_Content_Table_3Col.pptx
|   ├── SRA-MS-009_Content_Table_4Col.pptx
|   ├── SRA-MS-010_Content_Table_5Col.pptx
|   ├── SRA-MS-011_Two_Column.pptx
|   ├── SRA-MS-012_Quote_Full_Emerald.pptx
|   ├── SRA-MS-013_Transition_Split.pptx
|   └─ SRA-MS-014_Back_Cover_Split.pptx
|
├─ 02_Slide_Content/
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|   ├── SRA-SC-002_Copyright_Classification.docx
|   ├── SRA-SC-003_Curriculum_Overview.docx
|   ├── ... (all 73 slides as individual .docx files)
|   └─ SRA-SC-073_Back_Cover.docx
|
├─ 03_Images/
|   ├── Photography/
|   |   ├── P-001_Ancient_Egyptian_Papyrus.jpg
|   |   ├── P-002_Chinese_Bamboo_Cups.jpg
|   |   ├── P-003_Islamic_Manuscript.jpg
|   |   ├── P-004_Clinical_Environment.jpg
|   |   ├── P-005_Hand_Hygiene.jpg
|   |   ├── P-006_Sunnah_Point_Palpation.jpg
|   |   ├── P-007_Cup_Placement.jpg
|   |   ├── P-008_Islamic_Architecture.jpg
|   |   ├── P-009_Natural_Texture.jpg
|   |   ├── P-010_Dates_Black_Seed.jpg
|   |   ├── P-011_Emergency_Equipment.jpg
|   |   └─ P-012_Mark_Progression.jpg
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 - └─ T-003_Gold_Foil_Accent.png
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 - └─ I-002_Circulatory_System_Diagram.svg
 - └─ I-003_Heart_Chambers.svg
 - └─ I-004_Lymphatic_System.svg
 - └─ I-005_Fascia_Layers.svg
 - └─ I-006_Sunnah_Points_Map.svg
 - └─ I-007_9Step_Procedure_Flowchart.svg
 - └─ I-008_Risk_Stratification_Tree.svg
 - └─ I-009_Emergency_Response_Sequence.svg
 - └─ I-010_Evidence_Hierarchy_Pyramid.svg
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 - └─ E-002_Islamic_Scholar_Portrait.svg
 - └─ E-003_Prophetic_Medicine_Symbols.svg
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 - └─ IC-006_Warning_Triangle.svg
 - └─ IC-007_Book.svg
 - └─ IC-008_Scale.svg
 - └─ IC-009_Droplet.svg
 - └─ IC-010_Moon.svg
 - └─ IC-011_Leaf.svg
 - └─ IC-012_Clock.svg
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 - └─ IC-014_Check_Circle.svg
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 - └─ Al_Razi_Al_Hawi.pdf
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 - └─ Mahdavi_2015_Diabetes.pdf
 - └─ PMC7735689_MFD_Study.pdf
 - └─ Hadith_Collection/
 - └─ Sahih_al_Bukhari_References.pdf

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- | └─ Regulatory/
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 - | └─ Spaulding_Sterilisation_Classification.pdf
 - | └─ GDPR_Healthcare_Compliance.pdf
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 - | └─ V-002_Scarification_Technique.mp4
 - | └─ V-003_Emergency_Response_Drill.mp4
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 - | └─ AU-002_Module_2_Introduction.mp3
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- └─ 09_Handouts/
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 - | └─ SRA-HND-2026-HJM-003_Informed_Consent_Template.pdf
 - | └─ SRA-HND-2026-HJM-004_Treatment_Record_Form.pdf
 - | └─ SRA-HND-2026-HJM-005_Incident_Report_Form.pdf
 - | └─ SRA-HND-2026-HJM-006_Emergency_Quick_Reference.pdf
 - | └─ SRA-HND-2026-HJM-007_Aftercare_Instructions.pdf
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- └─ 10_Workbook/
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 - | └─ SRA-WKB-2026-HJM-003_Case_Study_Template.pdf
 - | └─ SRA-WKB-2026-HJM-004_Reflection_Journal.pdf
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- └─ 11_Instructor_Guide/
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 - | └─ SRA-INS-2026-HJM-004_Discussion_Prompts.pdf
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 - | └─ SRA-STU-2026-HJM-004_Exam_Preparation.pdf
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 - | └─ Sunnah_Remedies_Logo_Monochrome.svg

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|   |─ SRA-PROD-2026-PROD-006-v1.0_QA_Checklist.pdf
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|       |─ SRA-DOC-2026-HJM-001-v1.0_Slides_Only.pdf
|       |─ SRA-DOC-2026-HJM-001-v1.0_Notes_Pages.pdf
```

APPENDIX A: FILE NAMING CONVENTION

All files follow the Sunnah Remedies Academy Document Control System:

SRA-[TYPE]-[YEAR]-[SERIES]-[NUMBER]-v[VERSION].[EXT]

Component	Meaning	Example
SRA	Sunnah Remedies Academy	—
TYPE	Document type (DOC, PROD, HND, WKB, INS, STU)	PROD = Production
YEAR	Full year (2026)	2026
SERIES	Document series (HJM = Hijama, BRD = Brand, etc.)	HJM
NUMBER	Sequential number within series	001
VERSION	Version number (v1.0, v1.1, v2.0)	v1.0

APPENDIX B: VERSION HISTORY

Version	Date	Author	Changes
v1.0	2026-07-06	Production Director	Initial production per Academy Blueprint v2.0 and Three Foundational Constitutions

END OF PRODUCTION PACKAGE

Sunnah Remedies Academy — Institute of Prophetic Medicine
Healing by the Book and the Sunnah
www.sunnahremedies.academy